

Medicare Benefit Policy Manual

Chapter 9 - Coverage of Hospice Services Under Hospital Insurance

Table of Contents
(Rev. 13664; Issued: 03-05-26)

Transmittals for Chapter 9

10 - Requirements - General

20 - Certification and Election Requirements

20.1 - Timing and Content of Certification

20.2 - Election, Revocation, and Discharge

20.2.1 - Hospice Election

20.2.1.1 – Hospice Election_Statement

20.2.1.2 Hospice Election Statement Addendum

20.2.1.3 Hospice Notice of Election

20.2.2 - Hospice Revocation

20.2.3 - Hospice Discharge

20.2.4-Hospice Notice of Termination or Revocation

20.3 - Election by Skilled Nursing Facility (SNF) and Nursing Facilities (NFs)
Residents and Dually Eligible Beneficiaries

20.4 - Election by Managed Care Enrollees

30 - Coinsurance

30.1 - Drugs and Biologicals Coinsurance

30.2 - Respite Care Coinsurance

40 - Benefit Coverage

40.1 - Covered Services

40.1.1 - Nursing Care

40.1.2 - Medical Social Services

40.1.3 - Physicians' Services

40.1.3.1 - Attending Physician Services

40.1.3.2 - Nurse Practitioners as Attending Physicians

- 40.1.3.3 - Physician Assistants as Attending Physicians
 - 40.1.4 - Counseling Services
 - 40.1.5 - Short-Term Inpatient Care
 - 40.1.6 - Medical Appliances and Supplies
 - 40.1.7 - Hospice Aide and Homemaker Services
 - 40.1.8 - Physical Therapy, Occupational Therapy, and Speech-Language Pathology
 - 40.1.9 - Other Items and Services
- 40.2 - Special Services
 - 40.2.1 - Continuous Home Care (CHC)
 - 40.2.2 - Respite Care
 - 40.2.3 - Bereavement Counseling
 - 40.2.4 - Special Modalities
- 40.3 - Contracting With Physicians
- 40.4 - Core Services
 - 40.4.1 - Contracting for Core Services
 - 40.4.1.1 - Contracting for Highly Specialized Nursing Services
 - 40.4.2 - Waiver for Certain Core Staffing Requirements
 - 40.4.2.1 - Waiver for Certain Core Nursing Services
- 40.5 - Non-Core Services
- 50 - Limitation on Liability for Certain Hospice Coverage Denials
- 60 - Provision of Hospice Services to Medicare/Veteran's Eligible Beneficiaries
- 70 – Hospice Contracts with An Entity for Services not Considered Hospice Services
 - 70.1 - Instructions for the Contractual Arrangement
- 80 – Hospice Pre-Election Evaluation and Counseling Services
 - 80.1 – Documentation
 - 80.2 - Payment
- 90 - Caps and Limitations on Hospice Payments
 - 90.1 - Limitation on Payments for Inpatient Care
 - 90.2 - Aggregate Cap on Overall Reimbursement to Medicare-certified Hospices
 - 90.2.1 - New Hospices
 - 90.2.2 - Counting Beneficiaries for Calculation
 - 90.2.3 - Changing Aggregate Cap Calculation Methods

90.2.4 - Other Issues

90.2.5 - Updates to the Cap Amount

90.3 - Administrative Appeals

10 - Requirements - General

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

Hospice care is a benefit under the hospital insurance program. To be eligible to elect hospice care under Medicare, an individual must be entitled to Part A of Medicare and be certified as being terminally ill. An individual is considered to be terminally ill if the medical prognosis is that the individual's life expectancy is 6 months or less if the illness runs its normal course. Only care provided by (or under arrangements made by) a Medicare certified hospice is covered under the Medicare hospice benefit.

The hospice admits a patient only on the recommendation of the medical director in consultation with, or with input from, the patient's attending physician (if any).

In reaching a decision to certify that the patient is terminally ill, the hospice medical director must consider at least the following information:

- (1) Diagnosis of the terminal condition of the patient.
- (2) Other health conditions, whether related or unrelated to the terminal condition.
- (3) Current clinically relevant information supporting all diagnoses.

Section 1814(a)(7) of the Social Security Act (the Act) specifies that certification of terminal illness for hospice benefits shall be based on the clinical judgment of the hospice medical director or physician member of the interdisciplinary group (IDG) and the individual's attending physician, if he/she has one, regarding the normal course of the individual's illness. No one other than a medical doctor or doctor of osteopathy can certify or re-certify a terminal illness. Predicting of life expectancy is not always exact. The fact that a beneficiary lives longer than expected in itself is not cause to terminate benefits. "Attending physician" is further defined in section 20.1 and 40.1.3.1.

An individual (or his authorized representative) must elect hospice care to receive it. The first election is for a 90-day period. An individual may elect to receive Medicare coverage for two 90-day periods, and an unlimited number of 60-day periods. If the individual (or authorized representative) elects to receive hospice care, he or she must file an election statement with a particular hospice. Hospices obtain election statements from the individual and file a Notice of Election with the Medicare contractor, which transmits them to the Common Working File (CWF) in electronic format. Once the initial election is processed, CWF maintains the beneficiary in hospice status until a final claim indicates a discharge (alive or due to death) or until an election termination is received.

For the duration of the election of hospice care, an individual must waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the hospice designated by the individual (unless provided under arrangements made by the designated hospice); and

- Any Medicare services that are related to the treatment of the terminal condition for which hospice care was elected or a related condition, or services that are equivalent to hospice care, except for services provided by:
 1. The designated hospice (either directly or under arrangement);
 2. Another hospice under arrangements made by the designated hospice; or
 3. The individual's attending physician, who may be a nurse practitioner (NP) or a physician assistant (PA), if that physician, NP, or PA is not an employee of the designated hospice or receiving compensation from the hospice for those services.

Medicare services for a condition completely unrelated to the terminal condition for which hospice was elected remain available to the patient if he or she is eligible for such care.

20 - Certification and Election Requirements

(Rev. 1, 10-01-03)

A3-3141, HO-204

20.1 - Timing and Content of Certification

(Rev: 13664; Issued: 03-05-26; Effective: 10-01-25; Implementation: 04-06-26)

For the first 90-day period of hospice coverage, the hospice must obtain, no later than 2 calendar days after hospice care is initiated, (that is, by the end of the third day), oral or written certification of the terminal illness by the medical director of the hospice or the physician member of the hospice IDG, and the individual's attending physician if the individual has an attending physician.

No one other than a medical doctor or doctor of osteopathy can certify or re-certify an individual as terminally ill, meaning that the individual has a medical prognosis that his or her life expectancy is 6 months or less if the illness runs its normal course. Nurse practitioners and physician assistants cannot certify or re-certify an individual as terminally ill. In the event that a beneficiary's attending physician is a nurse practitioner or a physician assistant, the hospice medical director or the physician member of the hospice IDG certifies the individual as terminally ill.

The attending physician is a doctor of medicine or osteopathy who is legally authorized to practice medicine or surgery by the state in which he or she performs that function, a nurse practitioner, or physician assistant, and is identified by the individual, at the time he or she elects to receive hospice care, as having the most significant role in the determination and delivery of the individual's medical care. A nurse practitioner is defined as a registered nurse who performs such services as legally authorized to perform (in the state in which the services are performed) in accordance with State law (or State regulatory mechanism provided by State law) and who meets training, education, and

experience requirements described in 42 CFR 410.75. A PA is defined as a professional who has graduated from an accredited physician assistant educational program who performs such services as he or she is legally authorized to perform (in the State in which the services are performed) in accordance with State law (or State regulatory mechanism provided by State law) and who meets the training, education, and experience requirements as the Secretary may prescribe. The PA qualifications for eligibility for furnishing services under the Medicare program can be found in the regulations at 42 CFR 410.74 (c).

Note that a rural health clinic (RHC) or federally qualified health center (FQHC) physician can be the patient's attending physician. Refer to Chapter 13, section 210 for more information.

Initial certifications may be completed up to 15 days before hospice care is elected. Payment normally begins with the effective date of election, which is the same as the admission date. If the physician forgets to date the certification, a notarized statement or some other acceptable documentation can be obtained to verify when the certification was obtained.

For the subsequent periods, recertifications may be completed up to 15 days before the next benefit period begins. For subsequent periods, the hospice must obtain, no later than 2 calendar days after the first day of each period, a written certification statement from the medical director of the hospice or the physician member of the hospice's IDG. If the hospice cannot obtain written certification within 2 calendar days, it must obtain oral certification within 2 calendar days. When making an oral certification, the certifying physician(s) should state that the patient is terminally ill, with a prognosis of 6 months or less. Because oral certifications are an interim step sometimes needed while all the necessary documentation for the written certification is gathered, it is not necessary for the physician to sign the oral certification. Hospice staff must make an appropriate entry in the patient's medical record as soon as they receive an oral certification.

The hospice must obtain written certification of terminal illness for each benefit period, even if a single election continues in effect. A written certification must be on file in the hospice patient's record prior to submission of a claim to the Medicare contractor. Clinical information and other documentation that support the medical prognosis must accompany the certification and must be filed in the medical record with the written certification. Initially, the clinical information may be provided verbally, and must be documented in the medical record and included as part of the hospice's eligibility assessment.

A complete written certification must include:

1. the statement that the individual's medical prognosis is that their life expectancy is 6 months or less if the terminal illness runs its normal course;

2. specific clinical findings and other documentation supporting a life expectancy of 6 months or less;
3. the signature(s) of the physician(s), the date signed, and the benefit period dates that the certification or recertification covers (for more on signature requirements, see Pub. 100-08, Medicare Program Integrity Manual, chapter 3, section 3.3.2.4).
4. as of October 1, 2009, the physician's brief narrative explanation of the clinical findings that supports a life expectancy of 6 months or less as part of the certification and recertification forms, or as an addendum to the certification and recertification forms;

If the narrative is part of the certification or recertification form, then the narrative must be located immediately above the physician's signature.

If the narrative exists as an addendum to the certification or recertification form, in addition to the physician's signature on the certification or recertification form, the physician must also sign immediately following the narrative in the addendum.

The narrative shall include a statement directly above the physician signature attesting that by signing, the physician confirms that he/she composed the narrative based on his/her review of the patient's medical record or, if applicable, his or her examination of the patient. The physician may dictate the narrative.

The narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients. The physician must synthesize the patient's comprehensive medical information in order to compose this brief clinical justification narrative.

For recertifications on or after January 1, 2011, the narrative associated with the third benefit period recertification and every subsequent recertification must include an explanation of why the clinical findings of the face-to-face encounter support a life expectancy of 6 months or less.

5. face-to-face encounter. For recertifications on or after January 1, 2011, a hospice physician or hospice nurse practitioner must have a face-to-face encounter with each hospice patient prior to the beginning of the patient's third benefit period, and prior to each subsequent benefit period. Failure to meet the face-to-face encounter requirements specified in this section results in a failure by the hospice to meet the patient's recertification of terminal illness eligibility requirement. The patient would cease to be eligible for the benefit.

The face to face encounter requirement is satisfied when the following criteria are met:

a. Timeframe of the encounter: The encounter must occur prior to the recertification for the third benefit period and each subsequent benefit period. The encounter must occur no more than 30 calendar days before the third benefit period recertification and each subsequent recertification. A face-to-face encounter may occur on the first day of the

benefit period and still be considered timely. (Refer to section 20.1.5.d below for an exception to this timeframe).

b. Attestation requirements: A hospice physician or nurse practitioner who performs the encounter must attest in writing that he or she had a face-to-face encounter with the patient, including the date of the encounter. The attestation, its accompanying signature, and the date signed, *may* be a separate and distinct section of, or an addendum to, the recertification form, and must be clearly titled. *The attestation requirement may also be fulfilled by not only a clearly titled section of or an addendum to the recertification form, but also by a signed and dated clinical note within the medical record that documents clear indication that the face-to-face encounter occurred and includes the date of the visit, the signature of the practitioner who conducted the face-to-face encounter, and the date of the signature.* Where a nurse practitioner or noncertifying hospice physician performed the encounter, *and* the attestation *is a separate and distinct section of, or an addendum to*, the recertification, *the attestation* must state that the clinical findings of that visit were provided to the certifying physician, for use in determining whether the patient continues to have a life expectancy of 6 months or less, should the illness run its normal course.

c. Practitioners who can perform the encounter: A hospice physician or a hospice nurse practitioner can perform the encounter. A hospice physician is a physician who is employed by the hospice or working under contract with the hospice. A hospice nurse practitioner must be employed by the hospice. A hospice employee is one who receives a W-2 from the hospice or who volunteers for the hospice. If the hospice is a subdivision of an agency or organization, an employee of that agency or organization assigned to the hospice is also considered a hospice employee. Physician Assistants (PAs), clinical nurse specialists, and outside attending physicians are not authorized by section 1814(a)(7)(D)(i) of the Act to perform the face-to-face encounter for recertification.

d. Timeframe exceptional circumstances for new hospice admissions in the third or later benefit period: In cases where a hospice newly admits a patient who is in the third or later benefit period, exceptional circumstances may prevent a face-to-face encounter prior to the start of the benefit period. For example, if the patient is an emergency weekend admission, it may be impossible for a hospice physician or NP to see the patient until the following Monday. Or, if CMS data systems are unavailable, the hospice may be unaware that the patient is in the third benefit period. In such documented cases, a face to face encounter which occurs within 2 days after admission will be considered to be timely. Additionally, for such documented exceptional cases, if the patient dies within 2 days of admission without a face to face encounter, a face to face encounter can be deemed as complete.

Recertifications that require a face-to-face encounter but which are missing the encounter are not complete. The statute requires a complete certification or recertification in order for Medicare to cover and pay for hospice services. Where the only reason the patient ceases to be eligible for the Medicare hospice benefit is the hospice's failure to meet the face-to-face requirement, Medicare would expect the hospice to discharge the patient

from the Medicare hospice benefit, but to continue to care for the patient at its own expense until the required encounter occurs, enabling the hospice to re-establish Medicare eligibility. The hospice can re-admit the patient to the Medicare hospice benefit once the required encounter occurs, provided the patient continues to meet all of the eligibility requirements and the patient (or representative) files an election statement in accordance with CMS regulations.

The hospice must file written certification statements and retain them in the medical record. Hospice staff must make an appropriate entry in the patient's medical record as soon as they receive an oral certification.

These requirements also apply to individuals who had been previously discharged during a benefit period and are being recertified for hospice care.

20.2 - Election, Revocation, and Discharge

(Rev. 209, Issued: 05-08-15, Effective: 10-01-14, Implementation: 05-04-15)

20.2.1 - Hospice Election

(Rev: 13664; Issued: 03-05-26; Effective: 10-01-25; Implementation: 04-06-26)

Each hospice designs and prints its election statement. The election statement must include the following items of information:

Identification of the particular hospice that will provide care to the individual;

The individual's or representative's (as applicable) acknowledgment that the individual has been given a full understanding of hospice care, particularly the palliative rather than curative nature of treatment;

The individual's or representative's (as applicable) acknowledgment that the individual understands that certain Medicare services are waived by the election;

The effective date of the election, which may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive;

The individual's designated attending physician (if any). Information identifying the attending physician recorded on the election statement should provide enough detail so that it is clear which physician, Nurse Practitioner (NP), or Physician Assistant (PA) was designated as the attending physician. This information should include, but is not limited to, the attending physician's full name, office address, NPI number, or any other detailed information to clearly identify the attending physician.

The individual's acknowledgment that the designated attending physician was the individual's or representative's choice.

The signature of the individual or representative.

An election to receive hospice care will be considered to continue through the initial election period and through the subsequent election periods without a break in care as long as the individual:

- (1) Remains in the care of a hospice;
- (2) Does not revoke the election; and
- (3) Is not discharged from the hospice.

For Medicare payment purposes, an election for Medicare hospice care must be made on or after the date that the hospice provider is Medicare-certified. As with any election, the hospice must fulfill all other admission requirements, such as certification or recertification, any required face-to-face encounters, or Conditions of Participation (CoP) assessments. See also Pub. 100-04, Medicare Claims Processing Manual, chapter 11, section 20.1.1.

An individual may change, once in each election period, the designation of the particular hospice from which he or she elects to receive hospice care. The change of the designated hospice is not considered a revocation of the election, but is a transfer. To change the designation of hospice programs, the individual must file, with the hospice from which he or she has received care and with the newly designated hospice, a signed statement that includes the following information:

- the name of the hospice from which the individual has received care,
- the name of the hospice from which they plan to receive care, and
- the date the change is to be effective. As described in Pub. 100-04, Medicare Claims

Processing Manual, chapter 11, section *20.1.3*, when a hospice patient transfers to a new hospice, *the admitting hospice submits a transfer notice after the transfer has occurred, and the beneficiary's hospice benefit is not affected*. The receiving hospice must complete all assessments required by the hospice conditions of participation as described in 42 CFR 418.54. Because the benefit period does not change in a transfer situation, if the patient is in the third or later benefit period and transfers hospices, a face-to-face encounter is not required if the receiving hospice can verify that the originating hospice had the encounter. A change of ownership of a hospice is not considered a change in the patient's designation of a hospice and requires no action on the patient's part. Medicare beneficiaries enrolled in managed care plans may elect hospice benefits. Federal regulations require that the Medicare contractor assigned the hospice specialty workload maintain payment responsibility for hospice services and may pay for other claims if that Medicare contractor is the geographically assigned Medicare contractor for the managed care enrollees who elect hospice; for specifics, see regulations at 42 CFR 417, Subpart P, 417.585, Special Rules: Hospice Care (b), and 42 CFR 417.531 Hospice Care Services (b). Institutional claims for services not related to the terminal illness would otherwise be the responsibility of another geographically assigned Medicare contractor.

Managed care enrollees who have elected hospice may revoke hospice election at any time, but claims will continue to be paid by fee-for-service Medicare contractors as if the beneficiary were a fee-for-service beneficiary until the first day of the month following the month in which hospice was revoked. As specified above, by regulation, the duration of payment responsibility by fee-for-service Medicare contractors extends through the remainder of the month in which hospice is revoked by hospice beneficiaries.

See Pub. 100-04, Medicare Claims Processing Manual, Chapter 2, “Admission and Registration” and Chapter 11, “Processing Hospice Claims,” for requirements for hospice reporting to the Medicare contractor.

20.2.1.1 - Hospice Election Statement

(Rev. 10437, Issued: 11-06-20, Effective: 10-01-20, Implementation: 12-09-20)

An individual who meets the eligibility requirements of § 418.20 may file an election statement with a particular hospice. If the individual is physically or mentally incapacitated, his or her representative (as defined in § 418.3) may file the election statement.

Each hospice designs and prints its election statement. The election statement must include the following items of information:

1. Identification of the particular hospice that will provide care to the individual;
2. The individual’s or representative’s (as applicable) acknowledgment that the individual has been given a full understanding of hospice care, particularly the palliative rather than curative nature of treatment;
3. The individual’s acknowledgement that the individual has been provided information on the hospice's coverage responsibility and that certain Medicare services are waived by the election. For hospice elections beginning on or after October 1, 2020, this would include providing the individual with information indicating that services unrelated to the terminal illness and related conditions are exceptional and unusual and the hospice should be providing virtually all care needed by the individual who has elected hospice;
4. The effective date of the election, which may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive;
5. The individual’s designated attending physician (if any). Information identifying the attending physician recorded on the election statement should provide enough detail so that it is clear which physician, Nurse Practitioner (NP), or Physician Assistant (PA) was designated as the attending physician. This information

should include, but is not limited to, the attending physician's full name, office address, NPI number, or any other detailed information to clearly identify the attending physician.

6. The individual's acknowledgment that the designated attending physician was the individual's or representative's choice.
7. For hospice elections beginning on or after October 1, 2020 the hospice must provide:
 - Information on individual cost-sharing for hospice services;
 - Notification of the individual's (or representative's) right to receive an election statement addendum if there are conditions, items, services, and drugs the hospice has determined to be unrelated to the individual's terminal illness and related conditions and would not be covered by the hospice;
 - Information on the Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO), including the right to immediate advocacy and BFCC-QIO contact information.
8. The signature of the individual or representative.

An election to receive hospice care will be considered to continue through the initial election period and through the subsequent election periods without a break in care as long as the individual:

- (1) Remains in the care of a hospice;
- (2) Does not revoke the election; and
- (3) Is not discharged from the hospice.

For Medicare payment purposes, an election for Medicare hospice care must be made on or after the date that the hospice provider is Medicare-certified. As with any election, the hospice must fulfill all other admission requirements, such as certification or recertification, any required face-to-face encounters, or Conditions of Participation (CoP) assessments. See also Pub. 100-04, Medicare Claims Processing Manual, chapter 11, section 20.1.1.

An individual may change, once in each election period, the designation of the particular hospice from which he or she elects to receive hospice care. The change of the designated hospice is not considered a revocation of the election, but is a transfer. To change the designation of hospice programs, the individual must file, with the hospice from which he or she has received care and with the newly designated hospice, a signed statement that includes the following information:

- the name of the hospice from which the individual has received care;
- the name of the hospice from which they plan to receive care; and
- the date the change is to be effective.

As described in Pub. 100-04, Medicare Claims Processing Manual, chapter 11, section 20.1.1, when a hospice patient transfers to a new hospice, the receiving hospice must file a new Notice of Election; however, the benefit period dates are unaffected. The receiving hospice must complete all assessments required by the hospice conditions of participation as described in 42 CFR 418.54. Because the benefit period does not change in a transfer situation, if the patient is in the third or later benefit period and transfers hospices, a face-to-face encounter is not required if the receiving hospice can verify that the originating hospice had the encounter.

A change of ownership of a hospice is not considered a change in the patient's designation of a hospice and requires no action on the patient's part.

Medicare beneficiaries enrolled in managed care plans may elect hospice benefits. Federal regulations require that the Medicare contractor assigned the hospice specialty workload maintain payment responsibility for hospice services and may pay for other claims if that Medicare contractor is the geographically assigned Medicare contractor for the managed care enrollees who elect hospice; for specifics, see regulations at 42 CFR 417, subpart P, 417.585, Special Rules: Hospice Care (b), and 42 CFR 417.531 Hospice Care Services (b). Institutional claims for services not related to the terminal illness would otherwise be the responsibility of another geographically assigned Medicare contractor.

Managed care enrollees who have elected hospice may revoke hospice election at any time, but claims will continue to be paid by fee-for-service Medicare contractors as if the beneficiary were a fee-for-service beneficiary until the first day of the month following the month in which hospice was revoked. As specified above, by regulation, the duration of payment responsibility by fee-for-service Medicare contractors extends through the remainder of the month in which hospice is revoked by hospice beneficiaries.

See Pub. 100-04, Medicare Claims Processing Manual, Chapter 2, "Admission and Registration" and Chapter 11, "Processing Hospice Claims," for requirements for hospice reporting to the Medicare contractor.

20.2.1.2 Hospice Election Statement Addendum

(Rev. 11056; Issued: 10-21-21; Effective: 10-01-21; Implementation: 12-22-21)

For Hospice elections beginning on or after October 1, 2020, in the event that the hospice determines there are conditions, items, services, or drugs that are unrelated to the individual's terminal illness and related conditions, the individual (or representative), non-hospice providers furnishing such items, services, or drugs, or Medicare contractors may request a written list as an addendum to the election statement.

If the election statement addendum is requested within 5 days from the date of a hospice election, then the hospice would have 5 days from that request date to furnish the addendum. If the addendum is requested during the course of hospice care (that is, 5 days after the effective date of the hospice election), the hospice must provide this

information, in writing, within 3 days of the request to the requesting individual (or representative), non-hospice provider, or Medicare contractor. If there are any changes to the content on the addendum during the course of hospice care, the hospice must update the addendum and provide these updates, in writing, to the individual (or representative).

If the beneficiary dies, revokes, or is discharged within the required timeframe after requesting the addendum (i.e., within 5 days or 3 days of the request, depending on when the request was made), and before the hospice has furnished the addendum, the addendum would not be required to be furnished, and this condition for payment would be considered satisfied. Likewise, if the beneficiary dies, revokes, or is discharged prior to signing the addendum (furnished within the required timeframe), no signature is required and this condition for payment would be considered satisfied.

The “date furnished” must be within the required timeframe (that is, 3 or 5 days of the beneficiary or representative request, depending on when such request was made), rather than the signature date. The hospice must include the “date furnished” on the addendum.

Only the beneficiary (or representative) is required to sign the addendum. The non-hospice provider is not required to sign the addendum, if they are the requesting entity. If a beneficiary (or representative) refuses to sign a requested addendum, the hospice must document clearly on the addendum the reason the addendum is not signed.

While the addendum is not submitted with hospice claims, it is a condition for payment if the beneficiary (or representative) has requested it. This condition for payment is satisfied when there is a beneficiary (or representative) request present, which is documented by a valid signed addendum in the requesting beneficiary’s medical record with the hospice. If the claim has been selected for medical review, and it is clear based on received documentation that the beneficiary requested but did not receive the addendum within the time period specified at 42 CFR 418.24(c), the failure to provide such addendum would result in a claims denial. However, the Medicare Administrative Contractor may request the addendum to accompany any additional documentation request to mitigate such denial. A denial resulting from a violation of this specific condition for payment would be limited to only the claim subject to review (that is, it would not invalidate the entire hospice election).

The election statement addendum must include the following:

1. The addendum must be titled “Patient Notification of Hospice Non-Covered Items, Services, and Drugs.”
2. Name of the hospice.
3. Individual's name and hospice medical record identifier.
4. Identification of the individual's terminal illness and related conditions.

5. A list of the individual's conditions present on hospice admission (or upon plan of care update) and the associated items, services, and drugs not covered by the hospice because they have been determined by the hospice to be unrelated to the terminal illness and related conditions.
6. A written clinical explanation, in language the individual (or representative) can understand, as to why the identified conditions, items, services, and drugs are considered unrelated to the individual's terminal illness and related conditions and not needed for pain or symptom management. This clinical explanation must be accompanied by a general statement that the decision as to whether or not conditions, items, services, and drugs are related is made for each patient and that the individual should share this clinical explanation with other health care providers from which they seek items, services, or drugs unrelated to their terminal illness and related conditions.
7. References to any relevant clinical practice, policy, or coverage guidelines
8. Information on the following:
 - i. Purpose of Addendum. The purpose of the addendum is to notify the individual (or representative), in writing, of those conditions, items, services, and drugs the hospice will not be covering because the hospice has determined they are unrelated to the individual's terminal illness and related conditions.
 - ii. Right to Immediate Advocacy. The addendum must include language that immediate advocacy is available through the Medicare Beneficiary and Family Centered Care-Quality Improvement Organization (BFCC-QIO) if the individual (or representative) disagrees with the hospice's determination.
9. Name and signature of the individual (or representative) and date signed, along with a statement that signing this addendum (or its updates) is only acknowledgement of receipt of the addendum (or its updates) and not necessarily the individual's (or representative's) agreement with the hospice's determinations. If the individual (or representative) refuses to sign a requested addendum, the hospice must document why (on the addendum itself) and it would become a part of the medical record.
10. The date the hospice furnished the addendum. The date furnished must be within the required timeframe (that is, 3 or 5 days of the beneficiary or representative request, depending on when such request was made).

Example: Mr. Brown elects hospice on December 1st and requests the addendum on December 3rd. The hospice must provide this information, in writing, to Mr. Brown

within 5 days of the request. Therefore, the addendum would be required to be provided to Mr. Brown on or before December 8th.

Example: Mrs. Smith's effective date of her hospice election was November 1st, but she did not request the election statement addendum on that date. On December 4th, Mrs. Smith requests the election statement addendum. Since Mrs. Smith requested the election statement addendum during the course of hospice care (that is, after the first 5 days of the hospice election date), the hospice must provide this information, in writing, within 3 days of her request. Therefore, the addendum would be required to be provided to Mrs. Smith on or before December 7th.

Example: Miss Jones requested the election statement addendum on May 1st, the effective date of her initial hospice election. Miss Jones died on May 3rd. Because Miss Jones died within the first 5 days from the start of hospice care and before the hospice was able to furnish the addendum, the addendum would not be required to be furnished after Miss Jones has died, and this condition for payment would be considered met.

20.2.1.3- Hospice Notice of Election

(Rev. 10437, Issued: 11-06-20, Effective: 10-01-20, Implementation: 12-09-20)

Upon electing the Medicare hospice benefit, the beneficiary waives the right to Medicare payment for any Medicare services related to the terminal illness and related conditions (i.e., the patient's prognosis) during a hospice election, except when provided by, or under arrangement by, the designated hospice or individual's attending physician if he or she is not employed by the designated hospice (42 CFR 418.24 (d)).

In order to establish the hospice election in the Medicare claims processing system, the hospice must submit a Notice of Election (NOE) as described in chapter 11, section 20.1.1 of the Medicare Claims Processing Manual (Pub.100-04). Prompt filing of the hospice NOE with the Medicare contractor is required to properly enforce this waiver and prevent inappropriate payments to non-hospice providers. The effective date of hospice election is the same as the hospice admission date.

Timely-filed hospice NOEs shall be filed within 5 calendar days after the hospice admission date. A timely-filed NOE is one that is submitted to and accepted by the Medicare contractor within 5 calendar days after the hospice election. The practical meaning of 'submitted to and accepted by the Medicare contractor' is that the NOE was not returned to the provider for correction.

Example: The date of hospice election is October 1st. A timely-filed NOE would be submitted and accepted by the Medicare contractor on or before October 6th.

In instances where a NOE is not timely-filed, Medicare shall not cover and pay for the days of hospice care from the hospice admission date to the date the NOE is submitted to, and accepted by, the Medicare contractor. These days shall be provider liable, and the provider shall not bill the beneficiary for them.

Example: The date of hospice election is October 1st. The NOE was not submitted and accepted by the Medicare contractor until October 10th. Provider liable days would be October 1st through October 9th.

There may be some circumstances that may be beyond the control of the hospice where it may not be possible to timely-file the NOE within 5 calendar days after the effective date of election or timely-file the Notice of Termination or Revocation (NOTR) (see section 20.2.4 - Hospice Notice of Termination or Revocation) within 5 calendar days after the effective date of a beneficiary's discharge or revocation. Therefore, the regulations do allow for exceptions. There are four circumstances that may qualify the hospice for an exception to the consequences of filing the NOE more than 5 calendar days after the effective date of election. These exceptional circumstances are as follows:

1. Fires, floods, earthquakes, or other unusual events that inflict extensive damage to the hospice's ability to operate;
2. An event that produces a data filing problem due to a CMS or Medicare contractor systems issue that is beyond the control of the hospice;
3. A newly Medicare-certified hospice that is notified of certification after the Medicare certification date, or is awaiting its user ID from its Medicare contractor; or,
4. Other circumstances determined by CMS to be beyond the control of the hospice.

If one of the four circumstances described above prevents a hospice from timely-filing its NOE, the hospice must document the circumstance to support a request for an exception, which would waive the consequences of filing the NOE late. Using that documentation, the hospice's Medicare contractor will determine if a circumstance encountered by a hospice qualifies for an exception to the consequences for filing an NOE more than 5 calendar days after the effective date of election. If the request for an exception is denied, the Medicare contractor will retain the decision of the denial. Hospices retain their usual appeal rights on the claim for payment.

A retroactive Medicare entitlement qualifies as one of the exceptions to a timely-filed NOE as this would be a circumstance that is beyond the hospice's control. An individual must be entitled to Medicare Part A in order to be eligible to receive services under the Medicare hospice benefit and an individual who receives retroactive Medicare entitlement is entitled to Medicare hospice services effective on the first day of that entitlement. In the event of retroactive Medicare entitlement, the hospice would submit a request for an exception, which would waive the consequences of filing the NOE late. To receive an exception, the individual must meet eligibility requirements under the Medicare hospice benefit and must have elected to receive services under the Medicare hospice benefit. Therefore, the hospice must be able to provide the following documentation to Medicare contractors and/or CMS, if requested:

- (1) Proof of retroactive Medicare entitlement;

- (2) The certification of terminal illness that meets the criteria set forth in section 20.1; and
- (3) The hospice election statement that meets the criteria set forth in section 20.2.1.1.

See Pub. 100-04, Medicare Claims Processing Manual, Chapter 11, “Processing Hospice Claims” for requirements for NOE submission, reporting provider-liable days, and qualifying circumstances for a request for exception.

20.2.2 - Hospice Revocation

(Rev: 13664; Issued: 03-05-26; Effective: 10-01-25; Implementation: 04-06-26)

An individual or representative may revoke the election of hospice care at any time in writing; however, a hospice cannot “revoke” a patient’s election. To revoke the election of hospice care, the individual must file a document with the hospice that includes:

- A signed statement that the individual revokes the election for Medicare coverage of hospice care for the remainder of that election period, and
- The effective date of that revocation. An individual may not designate an effective date earlier than the date that the revocation is made.

Note that a verbal revocation of benefits is NOT acceptable. The individual forfeits hospice coverage for any remaining days in that election period.

Upon revoking the election of Medicare coverage of hospice care for a particular election period, the individual is no longer covered under the Medicare hospice benefit and resumes Medicare coverage of the benefits waived when hospice care was elected. An individual may, at any time, elect to receive hospice coverage for any other hospice election periods that he or she is eligible to receive. *There is no waiting period for the hospice beneficiary to reelect the hospice benefit after revocation, and a subsequent election period can begin immediately after reelection if the individual continues to meet eligibility requirements.*

20.2.3 - Hospice Discharge

(Rev: 13664; Issued: 03-05-26; Effective: 10-01-25; Implementation: 04-06-26)

The hospice notifies the Medicare contractor of any discharge so that hospice services and billings are terminated as of that date. Upon discharge, the patient loses the remaining days in the benefit period. However, there is no increased cost to the beneficiary. General coverage under Medicare is reinstated at the time the patient revokes the benefit or is discharged. Once a hospice chooses to admit a Medicare beneficiary, it may not automatically or routinely discharge the beneficiary at its discretion, even if the care promises to be costly or inconvenient, or the State allows for discharge under State requirements. The election of the hospice benefit is the beneficiary’s choice rather than the hospice’s choice, and the hospice cannot revoke the beneficiary’s election. Neither should the hospice request or demand that the patient revoke his/her election. Discharge from a hospice can occur as a result of one of the following:

- The beneficiary decides to revoke the hospice benefit;
- The beneficiary transfers to another hospice;
- The beneficiary dies;
- The beneficiary moves out of the geographic area that the hospice defines in its policies as its service area. Some examples of moving out of the hospice's service area include, but are not limited to, when a hospice patient moves to another part of the country or when a hospice patient leaves the area for a vacation. Another example would be when a hospice patient is receiving treatment for a condition unrelated to the terminal illness or related conditions in a facility with which the hospice does not have a contract, and the hospice is unable to access the patient to provide hospice services. In this example, Medicare's expectation is that the hospice provider would consider the amount of time the patient is in that facility and the effect on the plan of care before making a determination that discharging the patient from the hospice is appropriate;
- The beneficiary's condition improves, and he/she is no longer considered terminally ill. In this situation, the hospice will be unable to recertify the patient. The beneficiary can ask the Quality Improvement Organization (QIO) for an expedited review of the discharge (see Pub. 100-04, chapter 30, section 260 for more information); or
- Discharge for cause: There may be extraordinary circumstances in which a hospice would be unable to continue to provide hospice care to a patient. These situations would include issues where patient safety or hospice staff safety is compromised. When a hospice determines, under a policy set by the hospice for the purpose of addressing discharge for cause, that the patient's (or other persons in the patient's home) behavior is disruptive, abusive, or uncooperative to the extent that delivery of care to the patient or the ability of the hospice to operate effectively is seriously impaired, the hospice can consider discharge for cause. The hospice must do the following before it seeks to discharge a patient for cause:
 - Advise the patient that a discharge for cause is being considered;
 - Make a serious effort to resolve the problem(s) presented by the patient's behavior or situation;
 - Ascertain that the patient's proposed discharge is not due to the patient's use of necessary hospice services; and
 - Document the problem(s) and efforts made to resolve the problem(s) and enter this documentation into the patient's medical records.

The hospice must notify the Medicare contractor and State Survey Agency of the circumstances surrounding the impending discharge. The hospice may also need to make referrals to other relevant state/community agencies (i.e., Adult Protective Services) as appropriate.

Discharge order: Prior to discharging a patient for any reason other than a patient revocation, transfer, or death, the hospice must obtain a written physician's discharge order from the hospice medical director. If a patient has an attending physician involved in his or her care, this physician should be consulted before discharge and his or her review and decision included in the discharge note.

Effect of discharge: An individual, upon discharge from the hospice during a particular election period for reasons other than immediate transfer to another hospice—

- Is no longer covered under Medicare for hospice care;
- Resumes Medicare coverage of the benefits waived; and
- May at any time elect to receive hospice care if he or she is again eligible to receive the benefit. *There is no waiting period for the hospice beneficiary to reelect the hospice benefit after discharge, and a subsequent election period can begin immediately after reelection if the individual continues to meet eligibility requirements.*

Discharge planning: The hospice must have in place a discharge planning process that takes into account the prospect that a patient's condition might stabilize or otherwise change such that the patient cannot continue to be certified as terminally ill. The discharge planning process must include planning for any necessary family counseling, patient education, or other services before the patient is discharged because he or she is no longer terminally ill.

Once a patient is no longer considered terminally ill with a life expectancy of 6 months or less if the disease runs its normal course, Medicare coverage and payment for hospice care should cease. Medicare does not expect that a discharge would be the result of a single moment that does not allow time for some post-discharge planning. Rather, it would be expected that the hospice's interdisciplinary group is following the patient, and if there are indications of improvement in the individual's condition such that hospice may soon no longer be appropriate, then planning should begin. If the patient seems to be stabilizing, and the disease progression has halted, then it could be the time to begin preparing the patient for alternative care. Discharge planning should be a process, and planning should begin before the date of discharge.

In some cases, the hospice must provide Advanced Beneficiary Notification (ABN) or a Notice of Medicare Non-Coverage (NOMNC) to patients who are being discharged. See Pub. 100-04, Medicare Claims Processing Manual, Chapter 30 "Financial Liability Protections", Section 50.15.3.1, for information on these requirements.

20.2.4 - Hospice Notice of Termination or Revocation

(Rev. 209, Issued: 05-08-15, Effective: 10-01-14, Implementation: 05-04-15)

Upon discharge or revocation of hospice care, the beneficiary immediately resumes the Medicare coverage that had previously been waived by the hospice election. As such, hospices should record the beneficiary's discharge or revocation in the claims processing system promptly.

If a hospice beneficiary is discharged alive or if a hospice beneficiary revokes the election of hospice care, the hospice shall submit a timely-filed Notice of Termination/Revocation (NOTR) unless the hospice has already filed a final claim. A timely-filed NOTR is a NOTR that is submitted to and accepted by the Medicare contractor within 5 calendar days after the effective date of discharge or revocation. Hospices continue to have 12 months from the date of service in which to file their claims.

See Pub. 100-04, Medicare Claims Processing Manual, Chapter 11, "Processing Hospice Claims" for requirements for NOTR submission.

20.3 - Election by Skilled Nursing Facility (SNF) and Nursing Facilities (NFs) Residents and Dually Eligible Beneficiaries

(Rev. 1, 10-01-03)

HO-204.2

A Medicare beneficiary who resides in an SNF or NF may elect the hospice benefit if:

- The residential care is paid for by the beneficiary; or
- The beneficiary is eligible for Medicaid and the facility is being reimbursed for the beneficiary's care by Medicaid, and
- The hospice and the facility have a written agreement under which the hospice takes full responsibility for the professional management of the individual's hospice care and the facility agrees to provide room and board to the individual.

A beneficiary could be in a SNF under the SNF benefit for a condition unrelated to the terminal condition and simultaneously be receiving hospice for the terminal condition.

The State Medicaid Agency pays the hospice the daily amount allowed by the State for room and board while the patient is receiving hospice care, and the hospice pays the facility. Room and board services include the performance of personal care services, assistance in activities of daily living, socializing activities, administration of medication, maintaining the cleanliness of a resident's room, and supervising and assisting in the use of durable medical equipment and prescribed therapies.

Whenever Medicaid is involved, the hospice sends a copy of the election form to the State Medicaid Agency at the time of election, and also notifies this agency when the patient is no longer receiving hospice care.

In States that offer the hospice benefit under the Medicaid program, dually eligible beneficiaries must elect the benefit under both programs at once.

20.4 - Election by Managed Care Enrollees

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

A managed care enrollee may elect the hospice benefit. After the hospice election, Medicare pays the hospice for hospice services and pays for services of the managed care attending physician, who may be a nurse practitioner, (as defined in section 20.1 of this chapter) and services not related to the patient's terminal illness, through the fee-for-service system. (See 42 CFR 417.531 and 417.585.) Once under a hospice election, a managed care patient may also choose to use a provider outside of his or her managed care organization for care unrelated to the terminal illness or related conditions, or as the attending physician. See Pub. 100-04, Medicare Claims Processing Manual, chapter 11, section 40 for requirements for physician billing.

Once a managed care enrollee has elected hospice, all his or her Medicare benefits revert to fee-for-service, though the enrollee still remains on managed care for any additional benefits provided by his or her managed care plan, such as dental or vision coverage. The Medicare hospice benefit, through fee-for-service Medicare, covers all hospice care from the effective date of election to the date of discharge or revocation. During the election, fee-for-service Medicare also covers attending physician services and all care unrelated to the terminal illness. Upon discharge or revocation, fee-for-service Medicare continues to cover the beneficiary through the end of the month when the beneficiary revokes or is discharged from hospice alive. At the start of the month following revocation or discharge, all billing and coverage revert back to the managed care plan (see Pub 100-04, Medicare Claims Processing Manual, chapter 11, §30.4).

30 - Coinsurance

(Rev. 1, 10-01-03)

A3-3142

Hospices may charge individuals for the applicable coinsurance amounts. An individual who has elected hospice care is liable for the following coinsurance payments.

30.1 - Drugs and Biologicals Coinsurance

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

Hospices are to provide all drugs and biologicals for the palliation and management of pain and symptoms of a patient's terminal illness and related conditions. An individual is liable for a coinsurance payment for each palliative drug and biological prescription furnished by the hospice while the individual is receiving routine home care or

continuous home care. The individual is not liable for any coinsurance for hospice-related drugs or biologicals provided while he or she is receiving general inpatient care or respite care.

The amount of coinsurance for each prescription approximates 5 percent of the cost of the drug or biological to the hospice, determined in accordance with the drug copayment schedule established by the hospice, except that the amount of coinsurance for each prescription may not exceed \$5.00. The cost of the drug or biological may not exceed what a prudent buyer would pay in similar circumstances. The drug copayment schedule must be periodically reviewed for reasonableness and approved by the A/B MAC (HHH) before it is used.

30.2 - Respite Care Coinsurance

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

The amount of coinsurance for each respite care day is equal to 5 percent of the payment made by Medicare for a respite care day. The amount of the individual's coinsurance liability for respite care during a hospice coinsurance period may not exceed the inpatient hospital deductible applicable for the year in which the hospice coinsurance period began.

The individual hospice coinsurance period begins on the first day an election is in effect for the beneficiary and ends with the close of the first period of 14 consecutive days on each of which an election is not in effect for the beneficiary.

Thus, if a beneficiary receives hospice care for three election periods consecutively (without a 2-week break), he or she is subject to a maximum coinsurance for respite care equal to the hospital inpatient deductible. Similarly, if a break between election periods exceeds 14 days, the maximum coinsurance for respite care doubles, triples, or quadruples (depending on the number of election periods used and the timing of subsequent elections).

EXAMPLE: Mr. Brown elected an initial 90-day period of hospice care. Five days after the initial period of hospice care ended, he began another period of hospice care under a subsequent election. Immediately after the period ended, he began a third period of hospice care. Mr. Brown received inpatient respite care during all three periods of hospice care. Since these election periods were not separated by 14 consecutive days, they constitute a single hospice coinsurance period. Therefore, a maximum coinsurance for respite care during all three periods of hospice care may not exceed the amount of the inpatient hospital deductible for the year in which the first period began.

40 - Benefit Coverage

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

To be covered, hospice services must meet all of the following requirements:

- They must be reasonable and necessary for the palliation and management of the terminal illness as well as related conditions; and
- The individual must elect hospice care in accordance with sections 20.2 – 20.4 of this chapter; and
- A plan of care must be established and periodically reviewed by the attending physician, the medical director, and the interdisciplinary group of the hospice program; and
- That plan of care must be established before hospice care is provided; and
- The services provided must be consistent with the plan of care; and
- A certification that the individual is terminally ill must be completed as set forth in section 20.1 of this chapter.

A nurse practitioner serving as an attending physician should participate as a member of the IDG that establishes and/or updates the individual's plan of care. The nurse practitioner may not serve as or replace the medical director or physician designee.

All services provided by the hospice must be in accordance with a patient's individualized plan of care that is established and updated by the hospice interdisciplinary group, in consultation with the patient's attending physician (if any). The individualized plan of care is a continually evolving document. As such, Medicare expects the plan of care to be initiated based upon the information gathered in the patient's initial assessment, and the plan of care will be expanded upon, as appropriate, based on the information that is gathered during the comprehensive assessment.

Provided the above coverage criteria are met, hospices are paid a per diem rate based on the number of days and level of care provided during the election period. Levels of care are defined as:

- Routine home care (refer to §40.2.1); A routine home care day is a day on which an individual who has elected to receive hospice care is at home and is not receiving continuous home care.
- Continuous home care (refer to §40.2.1); A continuous home care day is a day on which an individual who has elected to receive hospice care is not in an inpatient facility (hospital, SNF, or hospice inpatient unit) and receives hospice care consisting predominantly of nursing care on a continuous basis at home. Hospice aide or homemaker services or both may also be provided on a continuous basis. Continuous home care is only furnished during brief periods of crisis and only as necessary to maintain the terminally ill patient at home.

- Inpatient respite care (refer to §40.1.5 and §40.2.2); An inpatient respite care day is a day on which the individual who has elected hospice care receives care in an approved facility on a short-term basis for respite.
- General inpatient care (refer to §40.1.5); A general inpatient care day is a day on which an individual who has elected hospice care receives general inpatient care in an inpatient facility for pain control or acute or chronic symptom management which cannot be managed in other settings.

Hospices are expected to furnish the following services to the extent specified by the plan of care for the individual. The categories listed above are used in billing to describe the acuity of the services furnished. See Pub. 100-04, Medicare Claims Processing Manual, Chapter 11, "Processing Hospice Claims," for a description of billing procedures.

40.1 - Covered Services

(Rev. 1, 10-01-03)

A3-3143.1, HO-230.1

Appropriately qualified personnel must perform all services, but it is the nature of the service, rather than the qualification of the person who provides it, that determines the coverage category of the service. The following services are covered hospice services.

40.1.1 - Nursing Care

(Rev. 12400; Issued: 12-06-23; Effective: 01-01-24; Implementation: 01-02-24)

To be covered as nursing services, the services must require the skills of a registered nurse (RN), or a licensed practical nurse (LPN) or a licensed vocational nurse (LVN) under the supervision of a registered nurse, and must be reasonable and necessary for the palliation and management of the patient's terminal illness and related conditions.

Services provided by a nurse practitioner (NP) who is not the patient's attending physician, are included under nursing care. This means that, in the absence of an NP, an RN, LPN, or LVN would provide the service. Since the services are nursing, payment is encompassed in the hospice per diem rate and may not be billed separately regardless of whether the services are provided by an NP or an RN. The following are examples of some services that traditionally are provided by an RN, which could also be provided by an NP, for which separate payment is not made:

- a. A patient with a terminal illness of lung cancer complains of leg pain. In the absence of an NP, an RN would assess the patient.
- b. Assessment of pain and or symptoms to determine the need for medications, other treatments, continuous home care, general inpatient care etc. In the absence of an NP, an RN would assess the patient.
- c. Administration of medications through intravenous (e.g., PICC, central, etc.), intrathecal or any other means. In the absence of an NP, an RN would administer the medication.

- d. Family counseling. In the absence of an NP, an RN, social worker, counselor, marriage and family therapists (MFT) or mental health counselors (MHCs) would provide this service.
- e. Providing a home visit for assessment or provision of care to a patient who is not his/her patient. In the absence of the NP, the service would be provided by an RN, LPN or LVN. Therefore, the NP cannot bill separately for the service.

40.1.2 - Medical Social Services

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

Medical social services must be provided by a person who meets the criteria given in the Conditions of Participation at 42 CFR 418.114(b)(3).

Services of these professionals which may be covered include, but are not limited to:

1. Assessment of the social and emotional factors related to the patient's illness, need for care, response to treatment and adjustment to care;
2. Assessment of the relationship of the patient's medical and nursing requirements to the patient's home situation, financial resources and availability of community resources;
3. Appropriate action to obtain available community resources to assist in resolving the patient's problem (**NOTE:** Medicare does not cover the services of a medical social worker to complete or assist in the completion of an application for Medicaid because Federal regulations require the State to provide assistance in completing the application to anyone who chooses to apply for Medicaid.);
4. Counseling services that are required by the patient; and
5. Medical social services furnished to the patient's family member or caregiver on a short-term basis when the hospice can demonstrate that a brief intervention (that is, two or three visits) by a medical social worker is necessary to remove a clear and direct impediment to the effective palliation and management of the patient's terminal illness and related conditions. To be considered "clear and direct," the behavior or actions of the family member or caregiver must plainly obstruct, contravene, or prevent the patient's medical treatment. Medical social services to address general problems that do not clearly and directly impede treatment, as well as long-term social services furnished to family members, such as ongoing alcohol counseling, are not covered.

40.1.3 - Physicians' Services

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

A physician must perform physicians' services (as defined in 42 CFR 410.20(b)(1)(1)), except that the services of the hospice medical director or the physician member of the

interdisciplinary group must be performed by a doctor of medicine or osteopathy. Nurse practitioners may not serve as a medical director or as the physician member of the interdisciplinary group. Nurse practitioners may not bill for medical services other than those described in 40.1.3.2.

The hospice face-to-face encounter is an administrative requirement related to certifying the terminal illness as required in §1814(a)(7)(D)(i) of the Social Security Act (the Act). By itself, it is not billable, as it is considered administrative (see Pub. 100-04, Medicare Claims Processing Manual, chapter 11, §40.1.1). However, if a hospice physician, or a hospice nurse practitioner who is also the patient's attending physician, provides reasonable and necessary non-administrative patient care during the face-to-face visit, that portion of the visit would be billable. See section 40.1.3.2 below for additional requirements for billing physician services provided by NPs.

40.1.3.1 - Attending Physician Services

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

The attending physician is a doctor of medicine or osteopathy, a nurse practitioner, or a physician assistant and is identified by the individual, at the time he or she elects to receive hospice care, as having the most significant role in the determination and delivery of the individual's medical care.

The election statement must include the patient's choice of attending physician. The information identifying the attending physician should be recorded on the election statement in enough detail so that it is clear which physician, NP, or PA was designated as the attending physician. This information should include, but is not limited to, the attending physician's full name, office address, NPI number, or any other detailed information to clearly identify the attending physician. Hospices have the flexibility to include this information on their election statement in whatever format works best for them, provided the content requirements in 42 CFR 418.24(b) are met. The language on the election form should include an acknowledgement by the patient (or representative) that the designated attending physician was the patient's (or representative's) choice.

If a patient (or representative) wants to change his or her designated attending physician, he or she must follow a procedure similar to that which currently exists for changing the designated hospice. Specifically, the patient (or representative) must file a signed statement with the hospice that identifies the new attending physician in enough detail so that it is clear which physician, NP, or PA was designated as the new attending physician. This information should include, but is not limited to, the attending physician's full name, office address, NPI number, or any other detailed information to clearly identify the attending physician. The statement must include the date the change is to be effective, the date that the statement is signed, and the patient's (or representative's) signature, along with an acknowledgement that this change in the attending physician is the patient's (or representative's) choice. The effective date of the change in attending physician cannot be earlier than the date the statement is signed.

40.1.3.2 - Nurse Practitioners as Attending Physicians **(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)**

A nurse practitioner is defined as a registered nurse who is permitted to perform such services as legally authorized to perform (in the state in which the services are performed) in accordance with State law (or State regulatory mechanism provided by State law) and who meets training, education and experience requirements described in 42 CFR 410.75.

If a beneficiary does not have an attending physician or a nurse practitioner who has provided primary care prior to or at the time of the terminal illness, the beneficiary may choose to be served by either a physician or a nurse practitioner who is employed by the hospice. The beneficiary must be provided with a choice of a physician or a nurse practitioner.

Medicare pays for attending physician services provided by nurse practitioners to Medicare beneficiaries who have elected the hospice benefit and who have selected a nurse practitioner as their attending physician. This applies to nurse practitioners without regard to whether they are hospice employees.

Physician services provided by nurse practitioners may be billed to Medicare only if the:

- Nurse practitioner is the beneficiary's designated attending physician; and
- Services are medically reasonable and necessary; and
- Services are performed by a physician in the absence of the nurse practitioner; and
- Services are not related to the certification of terminal illness.

If the nurse practitioner is employed by the hospice, the hospice can bill Part A for physician services meeting the above criteria on a hospice claim. If the nurse practitioner is not employed by the hospice, the nurse practitioner can bill Part B for physician services meeting the above criteria.

Payment for nurse practitioner services is made at 85 percent of the physician fee schedule amount. Services that are duplicative of what the hospice nurse would provide are not separately billable.

Nurse practitioners cannot certify or re-certify an individual as terminally ill, meaning that the individual has a medical prognosis that his or her life expectancy is 6 months or less if the illness runs its normal course. In the event that a beneficiary's attending physician is a nurse practitioner or a physician assistant, the hospice medical director or the physician member of the hospice IDG certifies the individual as terminally ill.

Hospice nurse practitioners may conduct face-to-face encounters as described in §20.1(5) as part of the certification process, but are still prohibited by statute from certifying the terminal illness.

40.1.3.3 – Physician Assistants as Attending Physicians **(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)**

Effective January 1, 2019, Medicare will pay for medically reasonable and necessary services provided by physician assistants (PAs) to Medicare beneficiaries who have elected the hospice benefit and who have selected a PA as their attending physician. PAs are paid 85 percent of the fee schedule amount for their services as designated attending physicians.

A physician assistant is defined as a professional who has graduated from an accredited physician assistant educational program who performs such services as he or she is legally authorized to perform (in the State in which the services are performed) in accordance with State law (or State regulatory mechanism provided by State law) and who meets the training, education, and experience requirements as the Secretary may prescribe. The physician assistant qualifications for eligibility for furnishing services under the Medicare program can be found in the regulations at 42 CFR 410.74 (c).

If a beneficiary does not have an attending physician, a nurse practitioner, or physician assistant who has provided primary care prior to or at the time of the terminal illness, the beneficiary may choose to be served by either a physician or a nurse practitioner who is employed by the hospice. The beneficiary must be provided with a choice of a physician or a nurse practitioner.

Medicare pays for attending physician services provided by physician assistants to Medicare beneficiaries who have elected the hospice benefit and who have selected a physician assistant as their attending physician. This applies to physician assistants without regard to whether they are hospice employees.

Effective January 1, 2019, Medicare will pay for medically reasonable and necessary services provided by PAs to Medicare beneficiaries who have elected the hospice benefit and who have selected a PA as their attending physician. PAs are paid 85 percent of the fee schedule amount for their services as designated attending physicians.

Attending physician services provided by PAs may be separately billed to Medicare only if:

- The PA is the beneficiary's designated attending physician; and
- Services are medically reasonable and necessary; and
- Services would normally be performed by a physician in the absence of the PA, whether or not the PA is directly employed by the hospice; and
- Services are not related to the certification of terminal illness.

If the physician assistant is employed by the hospice, the hospice can bill Part A for physician services meeting the above criteria on a hospice claim. If the physician assistant is not employed by the hospice, the physician assistant can bill Part B for physician services meeting the above criteria. PAs are authorized to furnish physician

services under their State scope of practice, under the general supervision of a physician; therefore the regulations at 42 CFR 410.150(a)(15) require that payment for PA services may be made to the employer or contractor of a PA.

Payment for physician assistant services is made at 85 percent of the physician fee schedule amount. Services that are duplicative of what the hospice nurse would provide are not separately billable.

Since PAs are not physicians, as defined in 1861(r)(1) of the Act, they may not act as medical directors or physicians of the hospice or certify the beneficiary's terminal illness and hospices may not contract with a PA for their attending physician services as described in section 1861(dd)(2)(B)(i)(III) of the Act, which outlines the requirements of the interdisciplinary group as including at least one physician, employed by or under contract with the agency or organization. All of these provisions apply to PAs without regard to whether they are hospice employees.

Physician assistants cannot certify or re-certify an individual as terminally ill, meaning that the individual has a medical prognosis that his or her life expectancy is 6 months or less if the illness runs its normal course. In the event that a beneficiary's attending physician is a nurse practitioner or a physician assistant, the hospice medical director or the physician member of the hospice IDG certifies the individual as terminally ill.

The hospice face-to-face encounter must be performed by a hospice physician or hospice nurse practitioner. PAs may not perform the face-to-face encounter.

40.1.4 - Counseling Services

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

Counseling services are provided to the terminally ill individual and the family members or other persons caring for the individual at home. Counseling, including dietary counseling, may be provided both for the purpose of training the individual's family or other caregiver to provide care, and for the purpose of helping the individual and those caring for the individual to adjust to the individual's approaching death. Bereavement counseling is available to the patient and his or her immediate family to provide emotional, psychosocial, and spiritual support and services before and after the death of the patient and to assist with issues related to grief, loss, and adjustment for up to 1 year after the patient's death. Also, see §40.5 regarding waivers under certain conditions for making dietary counseling available.

40.1.5 - Short-Term Inpatient Care

(Rev. 10437, Issued: 11-06-20, Effective: 10-01-20, Implementation: 12-09-20)

Short-term inpatient care may be provided in a participating hospital, hospice inpatient unit, or a participating SNF or NF that additionally meets the special hospice standards regarding patient and staffing areas. Medicare payment cannot be made for inpatient hospice care provided in a VA facility to Medicare beneficiaries eligible to receive

Veteran's health services. Services provided in an inpatient setting must conform to the written plan of care. However, dually eligible veterans residing at home in their community may elect the Medicare hospice benefit. See § 60, Provision of Hospice Services to Medicare/Veteran's Eligible Beneficiaries.

Medicare covers two levels of inpatient care: respite care for relief of the patient's caregivers, and general inpatient care which is for pain control and symptom management. General inpatient care (GIP) may only be provided in a Medicare participating hospital, SNF, or hospice inpatient facility. Respite care may only be provided in a Medicare participating hospital or hospice inpatient facility, or a Medicare or Medicaid participating nursing facility.

General inpatient care is allowed when the patient's medical condition warrants a short-term inpatient stay for pain control or acute or chronic symptom management that cannot feasibly be provided in other settings.

General inpatient care under the hospice benefit is not equivalent to a hospital level of care under the Medicare hospital benefit. For example, a brief period of general inpatient care may be needed in some cases when a patient elects the hospice benefit at the end of a covered hospital stay. If a patient in this circumstance continues to need pain control or symptom management, which cannot be feasibly provided in other settings while the patient prepares to receive hospice home care, general inpatient care is appropriate.

Other examples of appropriate general inpatient care include a patient in need of medication adjustment, observation, or other stabilizing treatment, such as psycho-social monitoring. It is not appropriate to bill Medicare for general inpatient care days for situations where the individual's caregiver support has broken down unless the coverage requirements for the general inpatient level of care are otherwise met. For a hospice to provide and bill for the general inpatient level of care, the patient must require an intensity of care directed towards pain control and symptom management that cannot be managed in any other setting.

Respite care is short-term inpatient care provided to the individual only when necessary to relieve the family members or other persons who normally care for the individual at home. Respite care may be provided only on an occasional basis and may not be reimbursed for more than 5 consecutive days at a time. Payment for the sixth and any subsequent day of respite care is made at the routine home care rate, and the patient would be liable for room and board. Respite care cannot be provided to hospice patients who reside in a facility (such as a long term care nursing facility). Provision of respite care depends upon the needs of the patient and of the patient's caregiver, within the limitations given.

Several examples of appropriate respite care for a patient who does not reside in a facility include providing a few days for the caregiver to rest at home, to visit family, attend a wedding, or attend a graduation for a needed break, or providing a few days immediately following a GIP stay if the usual caregiver has fallen ill. See also, section 40.2.2.

Note that hospice inpatient care in an SNF or NF serves to prolong current benefit periods for general Medicare hospital and SNF benefits. This could potentially affect patients who revoke the hospice benefit.

If a hospice patient receives general inpatient care for 3 days or more in a hospital, and chooses to revoke hospice, then the 3-day stay (although not equivalent to a hospital level of care) would still qualify the beneficiary for covered SNF services.

40.1.6 - Medical Appliances and Supplies

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

Medical appliances and supplies may be provided, including drugs and biologicals. Only drugs as defined in §1861(t) of the Act and which are used primarily for the relief of pain and symptom control related to the individual's terminal illness are covered. This includes both prescription and over-the-counter drugs as defined in §1861(t) of the Act. Appliances may include covered durable medical equipment as described in 42 CFR 410.38 as well as other self-help and personal comfort items related to the palliation or management of the patient's terminal illness. Equipment is provided by the hospice for use in the patient's home while the patient is under hospice care. Medical supplies include those that are part of the written plan of care and that are for palliation and management of the terminal illness or related conditions.

40.1.7 - Hospice Aide and Homemaker Services

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11; Implementation: 03-23-11)

A hospice aide is a person who meets the requirements described in the Conditions of Participation. Hospice aides may provide personal care services. Aides may also perform household services to maintain a safe and sanitary environment in areas of the home used by the patient, such as changing the bed or light cleaning and laundering essential to the comfort and cleanliness of the patient. Hospice aides are assigned to a specific patient by a registered nurse who is a member of the interdisciplinary group. Written patient care instructions for a hospice aide must be prepared by a registered nurse who is responsible for the supervision of a hospice aide.

CMS' Conditions of Participation define a qualified homemaker as an individual who meets the requirements described in 42 CFR 418.202(g) and who has successfully completed hospice orientation addressing the needs and concerns of patients and families coping with a terminal illness. Homemaker services may include assistance in maintenance of a safe and healthy environment and services to enable the individual to carry out the plan of care.

40.1.8 - Physical Therapy, Occupational Therapy, and Speech-Language Pathology

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11; Implementation: 03-23-11)

Physical therapy, occupational therapy, and speech-language pathology services may be provided for purposes of symptom control or to enable the individual to maintain activities of daily living and basic functional skills. Also, see §40.5 regarding waivers available under certain conditions for provision of these services.

40.1.9 - Other Items and Services

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

Any other item or service which is included in the plan of care and for which payment may otherwise be made under Medicare, in accordance with title XVIII of the Social Security Act, is a covered service under the Medicare hospice benefit. The hospice is responsible for providing any and all services indicated in the plan of care as reasonable and necessary for the palliation and management of the terminal illness and related conditions.

The hospice Interpretive Guidelines for 42 CFR 418.54(a), published via a Survey and Certification letter (S & C 09-19, Advance Copy-Hospice Program Interpretive Guidance Version 1.1), require that the initial assessment be conducted in the location where hospice services will be provided. The plan of care is developed from that initial assessment and from the comprehensive assessment. Ambulance transports to a patient's home which occur on the effective date of the hospice election (i.e., the date of admission), would occur prior to the initial assessment and therefore prior to the plan of care's development. As such, these transports are not the responsibility of the hospice. Medicare will pay for ambulance transports of hospice patients to their home, which occur on the effective date of hospice election, through the ambulance benefit rather than through the hospice benefit. Ambulance transports of a hospice patient, which are related to the terminal illness and which occur after the effective date of election, are the responsibility of the hospice.

EXAMPLE:

A hospice determines that an existing patient's condition has worsened and has become medically unstable. An inpatient stay will be necessary for proper palliation and management of the condition. The hospice adds this inpatient stay to the plan of care and decides that, due to the patient's fragile condition, the patient will need to be transported to the hospital by ambulance. In this case, the ambulance service becomes a covered hospice service.

40.2 - Special Services

(Rev. 1, 10-01-03)

A3-3143.2, HO-230.3

40.2.1 - Continuous Home Care (CHC)

(Rev. 12400; Issued: 12-06-23; Effective: 01-01-24; Implementation: 01-02-24)

Continuous home care may be provided only during a period of crisis as necessary to maintain an individual at home. A period of crisis is a period in which a patient requires continuous care which is predominantly nursing care to achieve palliation or management of acute medical symptoms. If a patient's caregiver has been providing a skilled level of care for the patient and the caregiver is unwilling or unable to continue providing care, this may precipitate a period of crisis because the skills of a nurse may be needed to replace the services that had been provided by the caregiver. This type of care can also be given when a patient resides in a long term care facility. However, Medicare regulations do not permit CHC to be provided in an inpatient facility (a hospice inpatient unit, a hospital, or SNF).

The hospice must provide a minimum of 8 hours of nursing, hospice aide, and/or homemaker care during a 24-hour day, which begins and ends at midnight. This care need not be continuous, e.g., 4 hours could be provided in the morning and another 4 hours in the evening. In addition to the 8 hour minimum, the services provided must be predominantly nursing care, provided by either an RN, an LPN, or an LVN. Services provided by a nurse practitioner that, in the absence of a nurse practitioner, would be performed by an RN, LPN, or LVN, are nursing services and are paid at the same continuous home care rate. This means that more than half of the hours of care are provided by an RN, LPN, or LVN. Homemaker or hospice aide services may be provided to supplement the nursing care.

NOTE: When fewer than 8 hours of care are required, the services are covered as routine home care rather than continuous home care.

Nursing care in the hospice setting can include skilled observation and monitoring when necessary, and skilled care needed to control pain and other symptoms.

The development of the CHC rate included the daily costs of nursing, hospice aide, social worker, and therapy visits; drugs; supplies and equipment; and the average daily cost of the hospice IDG. However, the statute limits the billable CHC hours of direct patient care to care provided by a nurse, a homemaker, or a hospice aide. Medicare regulations require that an hourly payment be made. While in the majority of situations, one individual would provide continuous care during any given hour, there may be circumstances where the patient's needs require direct interventions by more than one covered discipline resulting in an overlapping of hours between the nurse and hospice aide. In these circumstances, the overlapping hours would be counted separately. The total hours paid cannot exceed 24 hours per day.

The hospice would need to ensure that these direct patient care services are clearly documented and are reasonable and necessary. Computation of hours of care should also reflect the total hours of direct care provided to an individual that support the care that is needed and required. This means that all nursing and aide hours should be included in the computation for CHC and when the aide hours exceed the nursing hours, CHC would be denied and routine payment will be made. The statutory definition of continuous home care is meant to include the full range of services needed to achieve palliation and management of acute medical situations. Deconstructing what is provided in order to

meet payment rules is not allowed. In other words, hospices cannot discount any portion of the hours provided in order to qualify for a continuous home care day.

Documentation of care, modification of the plan of care, and supervision of aides or homemakers would not qualify as direct care nor would these activities qualify as necessitating the services of more than one care provider. In addition, while the services provided by other disciplines such as medical social workers, pastoral counselors, MFT or MHCs are an integral part of the care provided to a hospice patient, these services are not included in the statutory definition of continuous care and are not counted towards total hours of continuous care. However, the services of social workers, pastoral counselors, MFT or MHCs would be expected during these periods of crisis, if warranted as part of hospice care, and are included in the provisions of routine hospice care.

The following are used to illustrate circumstances that may qualify as CHC. This list is not all-inclusive nor does it indicate that if a patient presents with similar situations, that it would constitute CHC.

1. Frequent medication adjustment to control symptoms/collapse of family support system

Situation A: The patient has had a central venous catheter inserted to provide access for continuous Fentanyl drip for pain control and for the administration of antiemetic medication to control continuous nausea and vomiting. The nurse spends 2 hours teaching the family members how to administer IV medications. She returns in the evening for 1 hour. The hospice aide provides 3 hours of care. The nurse spends 2 hours phoning physicians, ordering medications, documenting and revising the plan of care.

Determination: Despite 8 hours of service, this does not constitute CHC since 2 of the 8 hours were not activities related to direct patient care.

Situation B: The patient experiences new onset seizures. He continues to have episodes of vomiting. The nurse remains with the patient for 4 hours (10 AM – 2 PM) until the seizures cease. During that time she provides skilled care and family teaching. The patient's wife states she is unable to provide any more care for her husband. A hospice aide is assigned to the patient for monitoring for 24 hours, beginning at 2:00 PM, with a total of 8 hours of direct care in the first day. The nurse returns intermittently for a total of an additional 5 hours to administer medications, assess the patient and to relieve the aide for breaks. The social worker provides 3 hours of services to work with the patient's wife in identifying alternative methods to care for the patient.

Determination: This qualifies as a continuous home care day. This constitutes a medical crisis, including collapse of family structure. The caregiver has been providing skilled care and the change in the patient's

condition requires the nurse's interventions. Since there is no overlap in nursing care, 17 hours of care (i.e., 9 hours of nursing care and 8 hours of aide care) would be computed as CHC. The social worker hours would not be incorporated. If the caregiver had been providing custodial care and his medical crisis resolved within a short time frame, this situation would not have qualified as CHC.

2. Symptom management/rapid deterioration/imminent death

Situation A: 77-year-old patient with lung cancer whose caregiver is 80 years old. The caregiver has been caring for this patient for 4 months and is now exhausted and scared. The care provided consists of assisting with bathing, assisting the patient to ambulate, preparing meals, housekeeping and administering oral medications. Since the patient is dyspneic at rest, she requires assistance in all ADLs, which equates to 9 hours of assistance within a 24-hour period.

Determination: This would not qualify as CHC since there is little nursing care that requires a nurse. The patient would however be a candidate for an inpatient respite level of care.

Situation B: The patient's condition deteriorates. The patient now has circumoral cyanosis, respiratory rate of 44 and labored with intermittent episodes of apnea. The nurse performs a complete assessment and teaches the caregiver on methods to make the patient comfortable. The nurse returns twice within the 24 - hour period to assess the patient. She revises the plan of care after conferring with the patient's attending physician and with the hospice physician. The homemaker and hospice aide are sent to assist the caregiver. Within the 24-hour period, the direct care provided by the nurse equates to 3 hours, homemaker with 2 hours, and hospice aide of 6 hours.

Determination: Since only 3 of the 11 hours were skilled care requiring the services of a nurse, this would not constitute CHC. In this situation, the care required is not predominantly nursing but are comprised of services provided by a hospice aide. In addition, it would not be correct to discount any portion of the hospice aide's hours or to provide these services gratis in order to qualify for the CHC benefit.

Situation C: The next day, the patient's condition deteriorates further. She has increased periods of apnea and air hunger. In addition she is experiencing continuous vomiting and increasing pain. Her blood pressure is beginning to decrease and her respirations are increasing. The nurse remains at the patient's bedside for 4 hours while attempting to control her pain and symptoms. The hospice aide provides care during 1 hour of this period. The nurse leaves and the hospice aide remains at the bedside for 3

hours. The social worker comes and talks with the caregiver and remains for 1 hour. The nurse returns while the aide leaves. The nurse remains with the patient for 2 hours until she dies. The social worker returns and stays with the caregiver for 1 hour until the mortuary arrives.

Determination: The nurse provided 6 hours of direct skilled nursing care; the aide provided 4 hours of direct care resulting in a total of 10 hours of registered nurse and hospice aide care. Since at least 6 of the 10 hours were direct nursing care, and since nursing care was the predominant service provided during the 10 hours, the care meets the criteria for CHC. In addition, since the nurse and the aide provided direct care for the patient simultaneously, it would be appropriate to bill for each resulting in total of 10 billable hours. The patient received 12 hours of care. The 2 hours for the social worker are not counted towards the CHC hours.

Medicare's requirements for coverage of CHC are that at least 8 hours of predominantly nursing care are needed in order to manage an acute medical crisis as necessary to maintain the individual at home. When a hospice determines that a beneficiary meets the requirements for CHC, appropriate documentation must be available to support the requirement that the services provided were reasonable and necessary and were in compliance with an established plan of care in order to meet a particular crisis situation. This would include the appropriate documentation of the situation and the need for continuous care services consistent with the plan of care.

Continuous home care is only furnished during brief periods of crisis and covered only as necessary to maintain the terminally ill individual at home.

40.2.2 - Respite Care

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

Respite care is short-term inpatient care provided to the individual only when necessary to relieve the family members or other persons caring for the individual at home. Respite care may only be provided in a Medicare participating hospital or hospice inpatient facility, or a Medicare or Medicaid participating nursing facility. Respite care may be provided only on an occasional basis and may not be reimbursed for more than 5 consecutive days at a time. Respite care provided for more than 5 consecutive days at a time must be billed as routine home care for day 6 and beyond, and the patient may be liable for room and board charges for day 6 and beyond. See §40.1.5 for additional information.

40.2.3 - Bereavement Counseling

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11; Implementation: 03-23-11)

Bereavement counseling consists of counseling services provided to the individual's family before and after the individual's death. Bereavement counseling is a required

hospice service, provided for a period up to 1 year following the patients' death. It is not separately reimbursable.

Bereavement specifics are found in Pub. 100-07, State Operations Manual, Appendix M, 42 CFR 418.64(d)(1), L596

40.2.4 - Special Modalities

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

A hospice may use chemotherapy, radiation therapy, and other modalities for palliative purposes if it determines that these services are needed. This determination is based on the patient's condition and the individual hospice's care-giving philosophy. No additional Medicare payment may be made regardless of the cost of the services.

40.3 - Contracting With Physicians

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11; Implementation: 03-23-11)

Section 1861(dd)(2) of the Act allows hospices to contract for physician services. Medical directors and physician members of the IDG are not required to be employed by the hospice. These physicians can be "under contract" with the hospice. Although the Act does not specify what the terms of that contract must be, requirements at 42 CFR 418.64(a), 418.100(e), and 418.102(a) are applicable to hospice, as well as all other responsibilities under the hospice conditions of participation. Hospices retain professional management responsibilities for these services and must ensure that qualified persons furnish them in a safe and effective manner. All physician employees and those under contract must function under the supervision of the hospice medical director. Since nurse practitioners are not included in the definition of a physician, this section does not apply to nurse practitioners

40.4 - Core Services

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11; Implementation: 03-23-11)

With the exception of physician services, substantially all core services must be provided directly by hospice employees on a routine basis. These services must be provided in a manner consistent with acceptable standards of practice. The following are hospice core services:

- Physician services.
- Nursing services, (routinely available and/or on call on a 24-hour basis, 7 days a week) provided by or under the supervision of an RN functioning within a plan of care developed by the hospice IDG in consultation with the patient's attending physician, if the patient has one.
- Medical social services by a qualified social worker under the direction of a physician.

- Counseling (including, but not limited to, bereavement, dietary, and spiritual counseling) with respect to care of the terminally ill individual and adjustment to death. The hospice must make bereavement services available to the family and other individuals identified in the bereavement plan of care up to 1 year following the death of the patient.

The hospice may contract for physician services as specified in the Conditions of Participation.

40.4.1 - Contracting for Core Services

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

A hospice may use contracted staff, if necessary, to supplement hospice employees in order to meet the needs of patients under extraordinary or other non-routine circumstances.

Arrangements made by a hospice to furnish items or services must be such that receipt of payment by the hospice for the services relieves the beneficiary of liability or any other persons to pay for the services. Whether the services and items are furnished by the hospice itself or by another organization under arrangements made by the hospice, both must agree not to charge the patient for covered services and items and must agree to return money incorrectly collected.

A hospice that has a written agreement with another agency, individual, or organization to furnish any services under arrangement must retain administrative and financial management, and oversight of staff and services for all arranged services, to ensure the provision of quality care. Arranged services must be supported by written agreements that require that all services be--

- (1) Authorized by the hospice;
- (2) Furnished in a safe and effective manner by qualified personnel; and
- (3) Delivered in accordance with the patient's plan of care.

40.4.1.1 - Contracting for Highly Specialized Nursing Services

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

A hospice may contract for the services of a registered nurse if the services are highly specialized, provided non-routinely, and so infrequently that the provision of such services directly would be impracticable and prohibitively expensive. Highly specialized services are determined by the nature of the service and the nursing skill level required to be proficient in the service. For example, a hospice may need to contract with a pediatric nurse if it cares for pediatric patients infrequently and if employing a pediatric nurse

would be impracticable and expensive. Continuous care is not a highly specialized service, because while time intensive, it does not require highly specialized nursing skills.

40.4.2 - Waiver for Certain Core Staffing Requirements (Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

Hospices are prohibited from contracting with other hospices and non-hospice agencies on a routine basis for the provision of the core services of nursing, medical social services and counseling to hospice patients. A hospice may, however, enter into arrangements with another hospice program or other entity for the provision of these core services in extraordinary, exigent, or other non-routine circumstances. An extraordinary circumstance generally would be a short-term temporary event that was unanticipated. Examples of such circumstances might include unanticipated periods of high patient loads, caused by an unexpectedly large number of patients requiring continuous care simultaneously, temporary staffing shortages due to illness, receiving patients evacuated from a disaster such as a hurricane or a wildfire, or temporary travel of a patient outside the hospice's service area. The hospice that contracts for services must maintain professional management responsibility for all services provided under arrangement or contract at all times and in all settings. Regulations at 42 CFR 418.100(e) discuss the professional management responsibilities of the hospice for services provided under arrangement.

Hospices must maintain evidence of the extraordinary circumstances that required them to contract for the core services and comply with the following:

- (a) The hospice must ensure that contracted staff is providing care that is consistent with the hospice philosophy and the patient's plan of care and is actively participating in the coordination of all aspects of the patient's hospice care.

Hospices may not routinely contract for a specific level of care (e.g., continuous care) or for specific hours of care (e.g., evenings and week-ends).

40.4.2.1 - Waiver for Certain Core Nursing Services (Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

The Conditions of Participation allow CMS to waive the requirement that a hospice provide nursing services directly, if the hospice is located in a non-urbanized area. The location of a hospice that operates in several areas is considered to be the location of its central office. The hospice must provide evidence to CMS that it has made a good faith effort to hire a sufficient number of nurses to provide services. CMS may waive the requirement that nursing services be furnished by employees based on the following criteria:

- The location of the hospice's central office is in a non-urbanized area as determined by the Bureau of the Census.

- There is evidence that the hospice was operational on or before January 1, 1983, including the following:
 - Proof that the organization was established to provide hospice services on or before January 1, 1983;
 - Evidence that hospice-type services were furnished to patients on or before January 1, 1983; and
 - Evidence that hospice care was a discrete activity rather than an aspect of another type of provider's patient care program on or before January 1, 1983.
- By virtue of the following evidence that a hospice made a good faith effort to hire nurses:
 - Copies of advertisements in local newspapers that demonstrate recruitment efforts;
 - Job descriptions for nurse employees;
 - Evidence that salary and benefits are competitive for the area; and
 - Any other recruiting activities (e.g., recruiting efforts at health fairs and contacts with appropriate personnel at other providers in the area).

A waiver remains in effect for a 1-year period. A waiver may be extended for two additional 1-year periods. Prior to each additional year, the hospice must request the extension and certify that the employment market for appropriate personnel has not changed significantly since the initial waiver was granted if this is the case. No additional evidence is required with this certification.

Waiver requests and any extensions with supporting documentation must be sent to the regional office for review. Regional offices have the authority to review, and approve, or deny the waiver application.

40.5 - Non-core Services

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

In addition to the hospice core services (physician services, nursing services, medical social services, and counseling), the following services must be provided by the hospice, either directly or under arrangements, to meet the needs of the patient and family:

- Physical and occupational therapy and speech-language pathology services.

- Hospice aide services. A hospice aide employed by a hospice, either directly or under contract, must meet the qualifications required by §1891(a)(3) of the Act and implemented at 42CFR418.76.
- Homemaker services.
- Volunteers.
- Medical supplies (including drugs and biologicals on a 24-hour basis) and the use of medical appliances related to the terminal illness and related conditions.
- Short-term inpatient care (including respite care and interventions necessary for pain control and acute and chronic symptom management) in a Medicare/Medicaid participating facility.

Section 1861(dd)(5) of the Act allows CMS to permit certain waivers of the requirements that the hospice make physical therapy, occupational therapy, speech language pathology services, and dietary counseling available (as needed) on a 24-hour basis. CMS is also allowed to waive the requirement that hospices provide dietary counseling directly. These waivers are available only to an agency or organization that is located in an area which is not an urbanized area (as defined by the Bureau of Census) and that can demonstrate to CMS that it has been unable, despite diligent efforts, to recruit appropriate personnel.

50 - Limitation on Liability for Certain Hospice Coverage Denials (Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

Section 1879 of the Act provides beneficiaries with liability protections from unexpected charges for certain denied claims when items or services are furnished by Medicare Part A hospice providers. Hospice providers may also be protected from liability under §1879 of the Act when certain conditions apply to a claim denial. The limitation on liability protections applies when a hospice claim denial is expected because:

- the beneficiary is determined to be not “terminally ill” as defined in §1879(g)(2) of the Act;
- specific items or services billed separately from the hospice per diem, such as physician services, are not reasonable and necessary as defined in either §1862(a)(1)(A) or §1862(a)(1)(C); or
- the level of hospice care is determined to be not reasonable or medically necessary as defined in §1862(a)(1)(A) or §1862(a)(1)(C), specifically for the management of the terminal illness and/or related conditions.

A/B MACs (HHH) will apply the usual procedures of the limitation on liability provision when a claim denial is based upon one of these reasons. When limitation on liability

protections applies, the hospice provider must issue the Advance Beneficiary Notice of Noncoverage (ABN), Form CMS-R-131, per CMS guidelines in order to transfer liability to the beneficiary.

See Pub. 100-04, Medicare Claims Processing Manual, Chapter 30, "Financial Liability Protections," particularly Section 50, "Advance Beneficiary Notice of Noncoverage."

60 - Provision of Hospice Services to Medicare/Veteran's Eligible Beneficiaries

(Rev. 12696; Issued:06-25-24; Effective:03-25-24; Implementation: 03-25-24)

Medicare beneficiaries that are dually eligible veterans, and reside at home in their community may elect the Medicare Hospice Benefit and have hospice services paid for under the Medicare Hospice Benefit. See §1853(c) and 1814(d) of the Act.

Upon electing the Medicare hospice benefit, the beneficiary waives the right to Medicare payment for any Medicare services related to the terminal illness and related conditions during a hospice election. However, this does not preclude dually eligible veterans (i.e., eligible for both Medicare and Veterans benefits) from receiving services not included on the hospice plan of care, and which are furnished and paid under the beneficiary's VA benefits, in addition to Medicare hospice services. This may include care and support services that are unique to VA benefits and not typically provided by Medicare hospice agencies, for example, but not limited to, VA home-based primary care for illnesses other than the terminal illness. Any services that are included on the hospice plan of care must be provided and paid under Medicare.

If a dually eligible veteran, who had been receiving Medicare hospice services in his/her home, is admitted to a VA owned and operated inpatient facility, the beneficiary must revoke the Medicare hospice benefit. Medicare is not allowed to pay for services that another Federal provider or agency furnishes (§1862(a)(3) and 42 CFR 411.6).

Dually eligible veterans may elect to receive Medicare hospice services while residing in community nursing homes and state homes and have those services paid for under the Medicare hospice benefit. (This is similar to paying for hospice care if a beneficiary lives in a nursing facility. See §20.3.)

70 – Hospice Contracts with An Entity for Services not Considered Hospice Services

(Rev. 1, 10-01-03)

A-02-102

The law governing the provision of Medicare hospice services is found at §1861(dd) of the Act. This law specifies the services covered as hospice care and the conditions a hospice program must meet in order to participate in the Medicare program. One of the

conditions a hospice program must meet is that it be “primarily engaged” in providing hospice care and services to terminally ill individuals. The law further clarifies that “terminally ill individuals” are individuals having a “medical prognosis that their life expectancy is six months or less if the illness runs its normal course.” Although the law does not explicitly define its expectations for “primarily engaged,” CMS has interpreted it to mean exactly what it says, that a hospice provider must be primarily engaged in providing hospice care and services (§1861(dd)(2)(A)(i)). “Primarily” does not mean “exclusively.” This requirement does not preclude provision of non-hospice services to terminally ill individuals who are not hospice patients or services to individuals, who are not terminally ill, so long as the primary activity of the hospice is the provision of hospice services to terminally ill individuals.

The CMS recognizes that there may be circumstances in which another health care entity may wish to “purchase” some of the highly specialized staff time or services of a hospice to better meet the needs of its specific patient population. In these cases, the services are not “hospice” services in terms of Medicare payment but become part of the service package of the provider under whose care the patient is. Examples of such circumstances are provided below.

EXAMPLE 1:

A dually eligible Medicare/Medicaid beneficiary enrolled in the Program of All-Inclusive Care for the Elderly (PACE) program for approximately 2 years has been diagnosed with a life limiting terminal illness with a prognosis of six months or less. In the course of routine assessments, the PACE provider recognizes that the beneficiary would benefit from the specialized services of a pain management specialist or a grief counselor. The PACE provider would then enter into a contractual arrangement with a Medicare certified hospice to purchase these specialized services. The hospice provider would bill the PACE provider for the services, and the PACE provider would in turn pay the hospice provider directly. Neither provider type would be allowed to bill Medicare separately for the contracted services (which in this example are PACE services and included in the PACE provider’s capitated rate). In this example, the PACE provider would maintain a medical record on the patient and the hospice provider would submit any documentation related to the care of the PACE patient to the PACE provider.

EXAMPLE 2:

A Medicare beneficiary is receiving skilled services from a Medicare certified home health agency (HHA). The beneficiary has been diagnosed with a life limiting terminal illness, but chooses to continue curative treatments, thereby rendering him ineligible for the Medicare hospice benefit. The beneficiary is experiencing a period of intractable pain, and the HHA wishes to purchase specialized pain control services from the hospice provider. The HHA would then enter into a contractual arrangement with a Medicare certified hospice to purchase specialized nursing services. The hospice would bill the HHA and the HHA would pay the hospice provider directly. Neither provider type would be allowed to bill Medicare separately for the contracted services (which, in this

example, are home health services and therefore included in the HHA's episode payment). In this example, the HHA would maintain a medical record on the patient, and the hospice submits any documentation related to the pain management to the HHA.

EXAMPLE 3:

A Medicare beneficiary (non-dual eligible) resides in a skilled nursing facility (SNF) and has a diagnosis of Alzheimer's disease. The beneficiary's disease process has progressed to a stage in which he/she can no longer ingest food or fluids. The beneficiary's family has been approached by the SNF regarding the placement of a feeding tube and has been told, "their loved one may not live much longer." The family is struggling with this concept and has requested assistance from the SNF regarding hospice care and grief counseling. The SNF has provided information about the Medicare hospice benefit to the family, but the patient's legal representative has made a decision not to elect hospice care at this time. The SNF does not have a trained grief counselor or full-time social worker on staff, but has a business relationship with a local hospice and has requested the services of a pastoral or grief counselor. The SNF and hospice enter into a contractual arrangement for the provision of grief counseling to this beneficiary's family by a pastoral care counselor. The hospice provider would bill the SNF, and the SNF would pay the hospice provider directly. Neither provider type would be allowed to bill Medicare Part A or B separately for the pastoral care services (which in this example are included in the Medicare's Resource Utilization Group or RUG payments to the SNF). The SNF maintains the medical record on this patient and the hospice provider would submit any documentation related to the pastoral care services provided to the SNF.

In all of the examples provided above, the billing and payment for the services are between each of the providers. Medicare must not be billed separately for any of the contracted services referred to in the examples provided above.

70.1 - Instructions for the Contractual Arrangement (Rev. 1, 10-01-03)

A contractual agreement between both parties must be on file and available for review by the state survey agency responsible for conducting surveys on behalf of CMS to assess compliance with the relevant conditions of participation for the provider contracting for the hospice services. Where a PACE organization contracts with a hospice organization, the contract, which is reviewed by CMS, must meet the requirements specified in 42 CFR 460.70. The agreement must specify each of the services to be provided, the credentials required for any of the professionals providing the services, the billing method and payment amounts, and any required documentation.

80 - Hospice – Pre-Election Evaluation and Counseling Services (Rev. 28, Issued: 12-03-04, Effective: 01-01-05, Implementation: 01-03-05)

Effective January 1, 2005, section 512 of the MMA amends section 1812(a)(1)(5) of the Act which, provides for a one-time payment to be made to a hospice for evaluation and

counseling services furnished by a physician who is either the medical director of or employee of a hospice agency. In order to be eligible to receive this service, a beneficiary must:

- be determined to have a terminal illness (which is defined as having a prognosis of 6 months or less if the disease or illness runs its normal course;
- not have made a hospice election, and
- not previously received the pre-election hospice services
- Services under this benefit are comprised of:
 - evaluating the individual's need for pain and symptom management;
 - counseling the individual regarding hospice and other care options, and may include;
 - advising the individual regarding advanced care planning.

The services that comprise this benefit are currently available through other Medicare benefits. For example, evaluation and counseling are often provided by an individual's physician as well as by other sources such as discharge planners, case managers, social workers and nonphysician providers. Therefore, this service may not be reasonable and necessary for all individuals. To the extent that beneficiaries have already received Medicare-covered evaluation and counseling with respect to end-of-life care, the hospice pre-election benefit would seem duplicative. However, if a beneficiary or the beneficiary's physician deem it necessary to seek the expertise of a hospice medical director or physician employee, this benefit is available to assure that a beneficiary's end-of-life options for care and pain management are addressed.

Since the decision to utilize this benefit is determined by the beneficiary or the beneficiary's physician, the evaluation and counseling service may not be initiated by the hospice, that is, the entity receiving payment for the service. Payments by hospice agencies to physicians or others in a position to refer patients for services furnished under this provision may implicate the Federal anti-kickback statute.

If the beneficiary's physician is also the medical director or physician employed by a hospice or possesses expertise in the provision of palliative or hospice care, that physician already possesses the expertise necessary to furnish end-of-life services and will have received payment for these services through the use of evaluation and management codes.

For example:

A thoracic surgeon has diagnosed a patient hospitalized in an acute care facility, with end-stage lung cancer with a prognosis of 6 months or less, if the disease runs its normal course. The patient has been informed of this diagnosis. The physician, with the patient's concurrence, requests a consult by the hospital's palliative care team. The team meets with the patient, discusses options, evaluates the patient's pain and symptoms, and makes recommendations including hospice care. Utilization of the evaluation and consultation benefit would be duplicative.

A patient with terminal cervical cancer has been receiving aggressive curative care as an outpatient, which has not been successful. The patient's physician, nurse and social worker have discussed the possibility of hospice. The patient decides to seek information from a hospice. Utilization of the evaluation and consultation benefit would be appropriate.

Hospice A receives referrals from various physicians and facilities that the patients are certified as having a terminal illness and wish to elect the hospice benefit. Hospice A utilizes the evaluation and consultation benefit for every patient as a preliminary evaluation, prior to the actual election of the benefit. Utilization of the evaluation and consultation benefit would not be appropriate.

Nursing home B contacts Hospice C providing them with a list of patients that can be certified as having a terminal illness. The medical director of Hospice C makes "rounds" on these patients, many of whom are unable to communicate and whose symptoms are being managed well. Utilization of the evaluation and consultation benefit would not be appropriate.

A patient is being treated by a physician for end-stage COPD. The patient is experiencing distressing symptoms, but has not been able to make any definitive decision as to advanced directive decisions. The patient's physician feels that the expertise of the medical director in Hospice D would be able to provide recommendations as to symptom management and advance directive decisions. The medical director provides the evaluation and consultation services. The patient does not elect the hospice benefit, but is able to make determinations as to his wishes and the physician has recommendations to assist in his provision of care. Utilization of the evaluation and consultation benefit would be appropriate.

80.1 - Documentation

(Rev. 188, Issued: 05-01-14; Effective: 08-04-14; Implementation: 08-04-14)

If the beneficiary's physician initiates the request for the evaluation and counseling service, appropriate documentation guidelines should be followed, including the determination of the terminal illness. Since this provision is not a prerequisite of or part of the hospice benefit, certification of the terminal illness is not required. The request or referral should be in writing, and the hospice medical director or physician employee would be expected to provide a written note on the patient's medical chart as well as maintaining a written record of this service.

If the beneficiary initiates the request for the service, the hospice agency should maintain a written record of the service and communication with the beneficiary's physician, with the beneficiary's permission, would occur, to the extent necessary to ensure continuity of care.

80.2 - Payment

(Rev. 141, Issued: 03-02-11, Effective: 01-01-11: Implementation: 03-23-11)

Section 512(b) of the MMA amends Section 1814(i) of the Act and establishes payment for this service. The statute specifies that the payment will be made to the hospice for services provided by the hospice medical director or physician employed by the hospice. The provision of these services may not be delegated to other hospice personnel (i.e., nurse practitioners, registered nurses, social workers, etc.) and may not be furnished by a physician under contract with the hospice. CMS intends to monitor data regarding the use of this benefit.

Since the evaluation and counseling provision is not a service within the hospice benefit, payment for these services is not included in the hospice payment cap.

Payment to the hospice agency for the provision of this evaluation and counseling service is made using HCPCS code G0337. The national payment amounts for this service for FY 2005 was \$54.57. Future changes in the rate will be identified in the Physician Fee Schedules. See Pub 100-04, Medicare Claims Processing Manual, chapter 11, section 10.1, for claims processing instructions.

90 – Caps and Limitations on Hospice Payments

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

To ensure that hospice care does not exceed the cost of conventional care, there are two annual limits to hospice payments. The statute requires that hospice payments be limited by an inpatient cap and by an aggregate cap in any given cap year. The cap determinations are calculated on an annual basis. Any amounts in excess of either cap are considered to be overpayments, and must be repaid to Medicare. The hospice inpatient cap limits the total number of Medicare inpatient days to no more than 20 percent of a hospice's total Medicare hospice days. The hospice aggregate cap limits the total aggregate payments any individual hospice can receive in a cap year to an allowable amount, based on an annual per beneficiary cap amount & the number of beneficiaries served.

Medicare contractors complete the hospice cap determinations for the inpatient cap after the end of the cap year. Hospices must file their self-determined aggregate cap determination notice with their Medicare contractor no later than 5 months after the end of the cap year and remit any overpayment due at that time. The Medicare contractor then reconciles all payments at the final cap determination. If a provider fails to file its aggregate cap determination 5 months after the end of the cap year, payments to the

provider are suspended in whole or in part until the self-determined cap is filed with the Medicare contractor.

For the 2016 cap year and earlier, the cap year for the inpatient and aggregate cap runs from November 1st to October 31st. For the 2018 cap year and later, the cap year for both the inpatient and aggregate cap, as well as the timeframes in which beneficiaries and payments are counted for the purposes of determining each individual hospice's aggregate cap aligns with the federal fiscal year (i.e., October 1st to September 30th).

In the year of transition (2017 cap year), for the inpatient cap, the Medicare contractors will calculate the percentage of all hospice days of care that were provided as inpatient days (GIP care and respite care) from November 1, 2016 through September 30, 2017 (11 months). For the 2017 cap year, hospices using the patient-by-patient proportional method for their aggregate cap determinations should count beneficiaries from November 1, 2016 to September 30, 2017. For those hospices using the streamlined method for their aggregate cap determinations, hospices should count beneficiaries from September 28, 2016 to September 30, 2017, which is 12 months plus 3 days, in that cap year's calculation. For the counting of hospice payments, hospices using either the streamlined method or the patient-by-patient proportional method, hospices should count 11 months of payments from November 1, 2016 to September 30, 2017 for the 2017 cap year. For the 2018 cap year and later, hospices should count both beneficiaries and payments, regardless of whether the streamlined or the patient-by-patient proportional methods are used, from October 1 to September 30.

Hospice Aggregate Cap Timeframes for Counting Beneficiaries and Payments for the Alignment of the Cap Year with the Federal Fiscal Year

Cap year	Beneficiaries		Payments	
	Streamlined method	Patient-by-patient proportional method	Streamlined method	Patient-by-patient proportional method
2016	9/28/15-9/27/16	11/1/15-10/31/16	11/1/15-10/31/16	11/1/15-10/31/16
2017 (Transition Year)	9/28/16-9/30/17 (12 months & 3 days)	11/1/16-9/30/17 (11 months)	11/1/16-9/30/17 (11 months)	11/1/16-9/30/17 (11 months)
2018 and later	10/1-9/30	10/1-9/30	10/1-9/30	10/1-9/30

90.1 – Limitation on Payments for Inpatient Care

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

Payments to a hospice for inpatient care are subject to a limitation on the number of days of inpatient care furnished to Medicare patients. The total inpatient days reported for both general inpatient and inpatient respite care may not exceed 20% of the total Medicare days reported by the hospice for a cap year. This limitation is applied once each year, at the end of the hospice's "cap year." The inpatient cap is calculated by the Medicare contractor as follows:

1. The maximum allowable number of inpatient days is calculated by multiplying the total number of days of Medicare hospice care by 0.20.
2. If the total number of days of inpatient care furnished to Medicare hospice patients is less than or equal to the maximum, no adjustment is necessary.
3. If the total number of days of inpatient care exceeds the maximum allowable number, the limitation is determined by:
 - Divide the maximum allowable inpatient days by total inpatient days reported on the Provider Statistical and Reimbursement Report (PS&R). Multiply the resulting ratio against total inpatient care reimbursement reported on the PS&R.
 - Multiply the excess inpatient care days by the routine home care (RHC) rate, wage adjusted for the location of the hospice.
 - Add together the amounts calculated in the two bullets above to derive the total allowable payments for inpatient care.
 - Compare the total allowable payments for inpatient care in bullet 3 above with actual payments made to the hospice for inpatient care during the “cap period” (i.e., the cap year) in order to determine the overpayments paid to the provider.

Any excess reimbursement must be refunded by the hospice.

EXAMPLE: Assume that:

40,000 total hospice days $\times$ 0.20 = 8,000 = the maximum allowable inpatient care days.

10,000 inpatient care days were reported and paid to the hospice.

The ratio of maximum allowable days to the number of actual days equals 8,000 to 10,000 or 0.80.

Assume the total reimbursement for inpatient care revenue codes 0655 and 0656 (representing Inpatient Respite Care and General Inpatient Care, respectively) for services provided between October 1st and September 30th is \$4,000,000.

$\$4,000,000 \times 0.80 = \$3,200,000$ = payments for allowable inpatient care days.

Excess inpatient days = (10,000 actual days) – (8,000 allowable days) = 2,000.
Multiply the excess inpatient care days by the routine home care rate of \$192.78,

wage adjusted for a hospice located in Redding, California, using the FY 2018 Wage Index value of 1.4968, leading to a wage-adjusted rate of \$288.55:

$2,000 \times \$288.55 = \$577,100$ = allowable payments for the excess inpatient care days.

Add the allowable inpatient payments and the allowable payments for excess days to derive the inpatient cap: $\$3,200,000 + \$577,100 = \$3,777,100$ = inpatient cap.

Compare \$3,777,100 inpatient cap with \$4,000,000 actually paid for inpatient revenue codes.

The hospice must refund $\$4,000,000 - \$3,777,100 = \$222,900$

If a provider's covered days of hospice care or Medicare payments are adjusted through an audit or other review, the Medicare contractor may recalculate the inpatient cap if the amount is material.

90.2 – Aggregate Cap on Overall Reimbursement to Medicare-certified Hospices

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

Overall aggregate Medicare payments made to a Medicare-certified hospice are subject to an aggregate cap for each cap year. The aggregate cap is calculated by multiplying a Medicare beneficiary count during the period by a statutory “cap amount.” The cap amount is adjusted annually. The Medicare beneficiary count is determined using either the proportional method or the streamlined method, as described in section 90.2.2 below.

The total actual Medicare payments made for services furnished to Medicare beneficiaries during the cap year are compared to the aggregate cap for this period. “Total actual Medicare payments made for services furnished to Medicare beneficiaries during the cap year” refers to Medicare payments for services rendered during the cap year, regardless of when payment is actually made. Any actual Medicare payments in excess of the aggregate cap must be refunded by the hospice.

All Medicare-certified hospices are subject to the aggregate cap calculation. When a beneficiary receives hospice care from more than one hospice, only the care provided by the Medicare-certified hospice(s) is considered when computing the aggregate cap.

90.2.1 – New Hospices

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

The hospice aggregate cap is calculated in a different manner for new hospices entering the Medicare program if the hospice has not participated in the program for an entire cap year. In this situation, the initial cap calculations for newly certified hospices must cover a period of at least 12 months but less than 24 months.

Hospices and Medicare contractors shall use the proportional method when calculating the aggregate cap for all hospices which are Medicare-certified on or after October 1, 2011 (as described in Section 90.2.2 below).

90.2.2 – Counting Beneficiaries for Calculation

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

Each hospice's cap amount is calculated by multiplying the adjusted cap amount by the number of Medicare beneficiaries who elected to receive hospice care from that hospice during the cap period.

The two methods for counting beneficiaries are the streamlined method and the proportional method, and are explained below.

Proportional Method: Under the proportional method, each hospice shall include in its number of Medicare beneficiaries only that fraction which represents the portion of a patient's total days of care in all hospices and all years that was spent in that hospice in that cap year, using the best data available at the time of the calculation (subject to revision at a later time based on updated data). The whole and fractional shares of Medicare beneficiaries' time in a given cap year are then summed to compute the total number of Medicare beneficiaries served by that hospice in that cap year.

The fractional share for any given beneficiary counted using the proportional method shall be calculated as follows:

Proportion = Beneficiary's Hospice Days in Cap Year in a Distinct Hospice /
Beneficiary's Total Hospice Days for all Years

When a hospice's cap is calculated using the proportional method, and a beneficiary included in that calculation survives into another cap year, the Medicare contractor may need to make adjustments to prior cap determinations. Reopening is allowed for up to 3 years from the date of the cap determination notice, except in the case of fraud, where reopening is unlimited. A revised cap determination letter issued as a result of a reopening may itself be reopened, subject to the 3 year limitation on reopening.

Streamlined Method: This method is used by eligible hospices that had their cap determinations calculated using the streamlined method for all cap years prior to cap year 2012 and elected to have their cap determination for cap years 2012 and beyond calculated using the streamlined method. The method that these hospices must use is described below.

- **When a beneficiary receives care from only one hospice:** Each beneficiary is counted as 1 in the first year of services with that hospice facility and will not be counted in any following years. The hospice includes in its number of Medicare

beneficiaries those Medicare beneficiaries who have not previously been included in the calculation of any hospice cap, and who have filed an election to receive hospice care during the timeframe for counting beneficiaries associated with the cap year, using the best data available at the time of the calculation.

Once a beneficiary has been included in the calculation of a hospice cap, he or she may not be included in the cap for that hospice again, even if the number of covered days in a subsequent cap year exceeds that of the period where the beneficiary was included (this could occur when the beneficiary has breaks between periods of election).

- **When a beneficiary receives care from more than one Medicare-certified hospice during a cap year or years:** Each Medicare-certified hospice includes in its number of Medicare beneficiaries only that fraction which represents the portion of a patient's total days of care in all Medicare-certified hospices and all years that was spent in that hospice in that cap year, using the best data available at the time of the calculation. Cap determinations are subject to reopening/adjustment to account for updated data. The streamlined method cap calculation for a Medicare beneficiary who has been in more than one Medicare-certified hospice is identical to the proportional method.

Beneficiary Counting Examples

The following examples are for illustrative purposes only.

As the examples illustrate, if the proportional method is applied for a given year, then every beneficiary who receives services in that year is counted based on the number of days of care furnished to the beneficiary in that year, relative to the total days of care for the beneficiary for all years.

Example 1. (One Hospice).

Jane Smith, a Medicare beneficiary, initially elected hospice care from Hospice A beginning on June 1, 2018. Her condition improved, and she was discharged from Hospice A on August 15, 2018, as she was no longer terminally ill. However, in November 2018 Ms. Smith's condition worsened; she re-elected hospice at Hospice A on November 15, 2018, and subsequently died on December 27, 2018.

Streamlined Method: Hospice A would count Ms. Smith as 1 in its 2018 cap year, but would not count Ms. Smith again in its 2019 cap year. Medicare payments for hospice care provided would be counted in the cap year in which those services were provided, regardless of when payments were actually made, using the best data available at the time of the calculation. Once a beneficiary has been included in the calculation of a hospice cap, he or she may not be included in the cap for that hospice again.

Proportional Method: Ms. Smith would be counted as follows:

2018 cap year (June 1 st – August 15 th):	76 days	=	76/119 =	0.64
2019 cap year (Nov 15 th – Dec 27 th):	43 days	=	43/119 =	0.36
Total days:	119 days	=		1.00

Hospices and Medicare contractors use the best data available at the time the cap is calculated to determine the proportional allocation of Ms. Smith's time. Because the Medicare contractor calculates the cap after allowing time for claims and adjustments to flow through the claims processing system, and assuming Hospice A files its claims without delay, by the time the 2018 cap is calculated the Medicare contractor would have information about Ms. Smith's complete hospice stay. Therefore, the Medicare contractor is able to correctly count Ms. Smith's stay for the 2018 and 2019 cap determinations, without having to make prior year adjustments to her proportional shares.

Example 2. (Reopening/Adjustment).

Continuing with the proportional method scenario from Example 1 above, had Ms. Smith lived until August 25, 2019, the hospice and the Medicare contractor would consider the information available at the time of the cap calculation when determining proportional shares.

For example, if the hospice calculated the 2018 cap on February 15, 2019, using claims for dates of service through December 31, 2018, Ms. Smith's total stay would have been 123 days, and the 2018 proportional share would be $76 / 123 = 0.62$.

Cap Accounting with Claims through December 31, 2018:

2018 cap year (June 1 st – August 15 th):	76 days =	$76/123 =$	0.62
2019 cap year (Nov 15 th – Dec 31 st):	47 days =	$47/123 =$	<u>0.38</u>
Total days:	123 days =		1.00

When calculating the 2019 cap determination using claims for dates of service through December 31, 2019, the Medicare contractor would be able to re-open the 2018 cap determination and correct the proportional allocation made in the previous cap year, to reflect a final allocation of $76/360 = 0.21$ for the 2018 cap determination and $284/360 = 0.79$ in the 2019 cap determination, reflecting the final date of August 25, 2019.

Cap Accounting with Claims through December 31, 2019:

2018 cap year (June 1 st – Aug 15 th):	76 days =	$76/360 =$	0.21
2019 cap year (Nov 15 th – Aug 25 th):	284 days =	$284/360 =$	<u>0.79</u>
Total days:	360 days =		1.00

Example 3. (Two Different Hospices).

Jane Smith, a Medicare beneficiary, initially elected hospice care from Hospice A beginning on June 1, 2018. Her condition improved, and she was discharged from Hospice A on August 15, 2018, as she was no longer terminally ill. However, in January 2019, Ms. Smith's condition worsened; she re-elected hospice at Hospice B on January 15, 2019, and subsequently died on February 26, 2019.

Streamlined Method:

The streamlined method cap calculation for a Medicare beneficiary who has been in more than one Medicare-certified hospice is identical to the proportional method (as described below). Therefore, Hospice A would count Ms. Smith as a fraction (0.64) in its 2018 cap year and would not count Ms. Smith again in its 2019 cap year. Hospice B would count Ms. Smith as a fraction (0.36) in its 2019 cap year and would not count Ms. Smith in its 2018 cap year. Medicare payments for hospice care provided would be counted in the cap year in which those services were provided, regardless of when payments were actually made, using the best data available at the time of the calculation.

Proportional Method:

Under the proportional method, Ms. Smith would be counted as follows:

Hospice A:

2018 cap year (June 1 st – August 15 th):	76 days =	$76/119 =$	0.64
--	-----------	------------	------

Hospice B

2019 cap year (Jan 15th - Feb 26th):	$\frac{43 \text{ days}}{119 \text{ days}} =$	$\frac{43}{119} =$	$\frac{0.36}{1.00}$
Total days:			

The Medicare contractor uses the best data available at the time the cap is calculated to determine the proportional allocation of Ms. Smith's time. Because the Medicare contractor calculates the cap after allowing time for claims and adjustments to flow through the claims processing system, and assuming Hospice A files its claims without delay, by the time the 2018 cap is calculated the Medicare contractor would have information about Ms. Smith's complete hospice stay. Therefore, the Medicare contractor is able to correctly count Ms. Smith's stay for the 2018 and 2019 cap determinations, without having to make prior year adjustments to her proportional shares.

90.2.3 – Changing Aggregate Cap Calculation Methods (Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

Hospices are not allowed to switch back and forth between cap calculation methods, as doing so would greatly complicate the cap determination calculation, would be difficult to administer, and could lead to inappropriate switching by hospices seeking merely to maximize Medicare payments. Additionally, in the year of a change in the calculation method or when a previous cap determination cannot be re-opened, there is a potential for over-counting some beneficiaries. Allowing hospices to switch back and forth between methods would perpetuate the risk of over-counting beneficiaries. Therefore:

- 1) Hospices that have their cap determination calculated using the proportional method for any cap year prior to the 2012 cap year will continue to have their cap calculated using the proportional method for the 2012 cap year and all subsequent cap years; and,
- 2) All other hospices would have their cap determinations for the 2012 cap year and all subsequent cap years calculated using the proportional method unless they make a one-time election to have their cap determinations for cap year 2012 and beyond calculated using the streamlined method. A/B MACs (HHH) do not reopen cap determinations for the 2011 cap year and prior cap years as a result of a hospice transition from the streamlined to the proportional method for the 2012 cap year. **NOTE:** this does not apply to hospices that appealed their cap determination.
- 3) A hospice would be able to elect the streamlined method no later than 60 days following the receipt of its 2012 cap determination.
- 4) Hospices which elected to have their cap determination calculated using the streamlined method may later elect to have their cap determinations calculated using the proportional method by either:
 - a. electing to change to the proportional method (if the election is made prior to receipt of the cap determination associated with the cap year where the change is desired); or

- b. appealing a cap determination calculated using the streamlined method to determine the number of Medicare beneficiaries.

5) If a hospice elected the streamlined method, and changed to the proportional method for a subsequent cap year, the hospice's aggregate cap determination for that cap year (i.e., the cap year of the change) and all subsequent cap years would be calculated using the proportional method. Past cap year determinations for the 2012 cap year and later cap years are subject to reopening; existing re-opening rules allow reopening for up to 3 years from the date of the cap determination, except in cases of fraud, where reopening is unlimited. A revised cap determination letter issued as a result of reopening may itself be reopened, subject to the 3 year limitation on reopening.

90.2.4 – Other Issues

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

The computation of the aggregate cap is made by the hospice after the cap year ends, which now is in alignment with the federal fiscal year.

Hospices can obtain instructions regarding the cap determination method election process from their Medicare contractor. Regardless of which method is used, the Medicare contractor shall continue to demand any additional overpayment amounts due to CMS at the time of the hospice cap determination. Cap determinations are subject to the existing CMS reopening regulations, which allow reopening for up to 3 years from the date of the cap determination letter, except in cases of fraud, where reopening is not limited.

If a provider's covered days of hospice care or Medicare payments are adjusted through an audit or other review, the Medicare contractor may recalculate the aggregate cap if the amount is material.

90.2.5 – Updates to the Cap Amount

(Rev. 11056; Issued: 10-21-21; Effective: 10-01-21; Implementation: 12-22-21)

The aggregate cap amount was set at \$6,500 per beneficiary when first enacted in 1983. Since 1983, the \$6,500 amount has been adjusted annually by the change in the medical care expenditure category of the consumer price index for urban consumers (CPI-U, United States city average), published by the Bureau of Labor Statistics, from March 1984 to the fifth month of the cap year, as required by section 1814(i)(2)(B) of the Act.

Section 1814(i)(2)(B)(i) and (ii) of the Act, as added by section 3(b) of the Improving Medicare Post-Acute Care Transformation Act (IMPACT Act) of 2014 (Pub. L. 113–185) requires, effective for the 2016 cap year (November 1, 2015 through October 31, 2016), that the cap amount for the previous year to be updated by the hospice payment update percentage, rather than the original \$6,500 being annually adjusted by the change in the CPI-U for medical care expenditures since 1984. This provision would have sunset for cap years ending after September 30, 2025, at which time the annual update to the cap amount would have reverted back to the original methodology. However,

division CC, section 404 of the Consolidated Appropriations Act, 2021 (CAA 2021) has extended the accounting years impacted by the adjustment made to the hospice cap calculation until 2030. Therefore, for accounting years that end after September 30, 2016 and before October 1, 2030, the hospice cap amount is updated by the hospice payment update percentage rather than using the CPI-U. This provision will sunset for cap years ending after September 30, 2030, at which time the annual update to the cap amount will revert back to the original methodology.

In those situations where a hospice begins participation in Medicare at any time other than the beginning of a cap year, and hence has an initial cap calculation for a period in excess of 12 months, a weighted average cap amount is used. The following example illustrates how this is accomplished.

EXAMPLE (Cap amounts utilized in this example are for illustrative purposes only and do not reflect actual cap amounts.)

09/01/18 - Hospice A is Medicare certified.

09/01/18 to 10/31/19 - First cap period (13 months) for hospice A.

Statutory cap amount for first Medicare cap year (09/01/18 - 09/30/18) = \$28,000.00

Statutory cap amount for second Medicare cap year
(10/01/18 - 09/30/19) = \$28,500.00

Weighted average cap amount calculation for hospice A:

One month (09/01/18 – 09/30/18) at \$28,000.00 = \$28,000.00

12 months (10/01/18 - 09/30/19) at \$28,500.00 = \$342,000.00

13 month period \$370,000.00 divided by 13 = \$28,461.54 (rounded)

In this example, \$28,461.54 is the weighted average cap amount used in the initial cap calculation for Hospice A for the period September 1, 2018, through September 30, 2019.

NOTE: If Hospice A had been certified in mid-month, a weighted average cap amount based on the number of days falling within each cap period is used.

90.3 – Administrative Appeals

(Rev. 246, Issued: 09-14-18, Effective: 12-17- 18, Implementation: 12-17-18)

The applicable Medicare contractor shall issue a Cap Determination Letter to notify hospice providers of the results of the Medicare contractor's cap calculations and to serve as the provider's Notice of Program Reimbursement (NPR). If there is a cap overpayment, there shall be an accompanying demand for repayment. As indicated in 42

CFR 418.311, a hospice that believes that its payments have not been properly determined may request a review from the applicable Medicare contractor or the Provider Reimbursement Review Board (PRRB). Each determination of program reimbursement shall include language describing the provider's appeal rights.

The above described letter, serving as the provider's determination of program reimbursement, shall include the following language:

“This notice is the Medicare contractor’s final determination for purposes of appeal rights. If you disagree with this determination, you may file an appeal, in accordance with 42 CFR 418.311 and 42 CFR, part 405, subpart R. The appeal should be filed with either the applicable Medicare contractor or the Provider Reimbursement Review Board (PRRB), depending on the amount in controversy. Appeal requests must be in writing and be filed within 180 days from the date of this determination.”

Transmittals Issued for this Chapter

Rev #	Issue Date	Subject	Impl Date	CR#
<u>R13664BP</u>	03/05/2026	Manual Updates Adding language to the Timing and Content of Certification, Revocation and Discharge Guidance, and Hospice Election	04/06/2026	14384
<u>R12696BP</u>	06/25/2024	Manual Updates for Clarification on Services Under the Medicare Hospice Benefit for Dually Eligible Veterans	03/25/2024	13523
<u>R12589BP</u>	04/19/2024	Manual Updates for Clarification on Services Under the Medicare Hospice Benefit for Dually Eligible Veterans- Rescinded and Replaced by Transmittal 12696	03/25/2024	13523
<u>R12516BP</u>	02/22/2024	Manual Updates for Clarification on Services Under the Medicare Hospice Benefit for Dually Eligible Veterans- Rescinded and Replaced by Transmittal 12589	03/25/2024	13523
<u>R12400BP</u>	12/06/2023	Hospice Benefit Policy Manual Updates Related to the Addition of Marriage and Family Therapists (MFTs) or Mental Health Counselors (MHCs) to the Hospice Interdisciplinary Team	01/02/2024	13437
<u>R12385BP</u>	11/30/2023	Hospice Benefit Policy Manual Updates Related to the Addition of Marriage and Family Therapists (MFTs) or Mental Health Counselors (MHCs) to the Hospice Interdisciplinary Team- Rescinded and Replaced by Transmittal 12400	01/02/2024	13437
<u>R11056BP</u>	10/22/2021	Manual Updates for Clarification on the Election Statement Addendum and Extension of the Hospice Cap Calculation Methodology	12/22/2021	12491
<u>R10437BP</u>	11/06/2020	Manual Updates Related to the Hospice Election Statement and the Implementation of the Election Statement Addendum	12/09/2020	12015
<u>R246BP</u>	09/14/2018	Manual Updates Related to Payment Policy Changes Affecting the Hospice Aggregate Cap Calculation and the Designation of Hospice Attending Physicians	12/17/2018	10517

<u>R209BP</u>	05/08/2015	Updates on Hospice Election Form, Revocation, and Attending Physician	05/04/2015	9114
<u>R205BP</u>	04/03/2015	Updates on Hospice Election Form, Revocation, and Attending Physician – Rescinded and replaced by Transmittal 209	05/04/2015	9114
<u>R188BP</u>	05/01/2014	Updates and Clarifications to the Hospice Policy Chapter of the Benefit Policy Manual	08/04/2014	8727
<u>R156BP</u>	06/01/2012	Updates to Caps and Limitations on Hospice Payments	07/02/2012	7838
<u>R141BP</u>	03/02/2011	New Hospice Certification Requirements and Revised Conditions of Participation (CoPs)	03/23/2011	7337
<u>R121BP</u>	02/05/2010	Medicare Systems Edits Refinements Related to Hospice Services	07/06/2010	6778
<u>R28BP</u>	12/03/2004	Hospice Pre-Election Evaluation and Counseling Services	01/03/2005	3585
<u>R22BP</u>	10/24/2004	Nurse Practitioners As Attending Physicians in the Medicare Hospice Benefit	06/28/2004	3226
<u>R15BP</u>	06/15/2004	Nurse Practitioners As Attending Physicians in the Medicare Hospice Benefit	N/A	3226
<u>R1BP</u>	10/01/2003	Introduction to the Benefit Policy Manual	N/A	N/A

[Back to top of Chapter](#)